PERSONAL STUDY GUIDE · NOVEMBER 2026 – JULY 2027

From UX Design to Counselling Psychology

The complete 9-month pre-admission guide: a week-by-week plan, 19 study modules, practice cases, admission prep and a glossary, all in one place.

Prepared for: a 28-year-old UX designer in India (B.E. Mechanical · HFI Certified Usability Analyst · ~₹8 LPA).
Destination: part-time MA Psychology (e.g. IGNOU MAPC, July 2027 intake) → counselling specialisation → supervised practice → NCAHP registration.
Focus areas: workplace counselling and digital-age wellbeing.
Pace: 3–5 hours a week, mostly listening, plus some reading and video.

What's inside

Part A	**Orientation**: the route, the money, how to use this guide, the study method
Part B	**The 36-week plan**: exactly what to do each week, with dates
Part C	**Study modules 1–19**: deep notes on everything you need before the degree Foundations (1–8) · The person & distress (9–11) · The craft of counselling (12–14) · Research (15) · Your niche (16–17) · Context & self (18–19)
Part D	**Practice**: 8 case vignettes, self-exercises, journal prompts, role-play scripts
Part E	**Admission**: IGNOU step by step, NCAHP check, alternatives, statement of purpose, interview prep
Part F	**Reference**: answers to self-checks, glossary of 80+ terms, full resource directory, sources

About the notes. Every study module was written specifically for this guide, in plain words. They are summaries, not text copied from books. Each module ends with links to the original or fuller sources. Use them to understand the ideas, then go deeper with the free textbook (*OpenStax Psychology 2e*) and the listed books and courses.

About facts that change. Fees, regulation (NCAHP/RCI) and admission dates were checked in October 2026 and are still changing. Always confirm on official websites before acting.

This guide is for learning, not treatment. If you, or anyone you're supporting, is in crisis, call **Tele-MANAS 14416** (free, 24×7, multiple Indian languages).

PART A

Orientation

Where you're going, what it costs, and how to study so it sticks.

A1. The route you've chosen

Route	Path from a B.E.	Time to practise	Fit
Psychiatrist	NEET-UG → MBBS (5.5 yrs) → NEET-PG → MD Psychiatry (3 yrs). Needs Class 12 Biology. No upper age limit for NEET-UG since 2022.	~10–11 yrs	Medical and prescription-focused. Not what you want.
Clinical Psychologist (RCI licence)	4-yr psychology UG (or 3-yr + 1-yr bridge) → 2-yr MA Clinical Psychology (replacing the M.Phil)	~6–7 yrs	Means restarting undergrad.
Counselling Psychologist	**MA/MSc Psychology (2 yrs, part-time possible) + supervised practice → NCAHP registration**	**~2.5–3 yrs**	**Your route.** Talk-based, person-centred, compatible with working.

Your timeline

When	Milestone
2 Nov 2026 – 11 Jul 2027	This guide: 36 weeks of self-study plus admission prep
Jul 2027	Enrol in part-time MA Psychology. Keep your UX job.
Jul 2027 – Jun 2028	Year 1: core courses MPC-001 to 006 plus practicals
Jul 2028 – Jun 2029	Year 2: counselling specialisation (theory, assessment, interventions, practicum, internship, project)
2029 – 2030	Extra supervised hours → NCAHP registration → start seeing clients part-time (evenings, weekends, online)
~2031 – 2033	Go full-time when practice income ≈ salary

The money picture

Item	Figure	Notes
Your salary today	~₹8 LPA (~₹67k/month)	—
IGNOU MAPC fee	~₹10k/yr (~₹20–21k total)	Aggregator figures; confirm on IGNOU portal
This 9-month guide	~₹2–4k (a few books)	Everything else listed is free
Whole transition (2–3 yrs)	~₹40k – ₹1 lakh	Incl. supervision, own therapy, books (estimate)
Salaried counsellor (early → mid)	~₹3–7 LPA → ₹5–10 LPA	Payscale early-career avg ≈ ₹3.25 LPA
Private practice per session	₹500–1,500 new; ₹2,000–3,500 experienced (metro)	Varies widely by city and niche
Sessions/week to match ₹8 LPA	~17 at ₹1,000 · ~11 at ₹1,500	Assuming 48 working weeks

> **Strategy: go hybrid.** Keep UX income through the degree. See clients part-time after registering, and switch fully when the numbers match. Your rare combination (designer plus counsellor) also opens higher-paying bridge roles at mental-health and workplace-wellbeing companies.

A2. How this guide works

The guide has one spine (**Part B, the 36-week plan**) and a body of knowledge (**Part C, 19 modules**). Each week of the plan tells you which module to study and what to listen to, watch and read alongside it.

Your weekly rhythm (3–5 hours)

Block	Time	What	When
🎧 Listen	~2 hrs	Podcast episodes or audiobook chapters	Commute, chores, walks
📖 Module	~1 hr	Read this week's module section in Part C, then skim the matching OpenStax chapter	Weekend morning
🎬 Watch	~45 min	Crash Course episodes or a Coursera week	One weekday evening
✍️ Reflect	~15 min	Notebook entry + the module's "Try this" exercise	Sunday evening

Busy week? Do only the listening and the notebook entry. Missing a week is fine; missing a month breaks the habit. Weeks 12, 25 and 35 are lighter review weeks with slack built in.

The core free resources you'll use throughout

FREE TEXTBOOK **OpenStax: Psychology 2e**. A complete university intro textbook, free online or as a PDF. Chapters: 1 Intro · 2 Research · 3 Biopsychology · 4 Consciousness · 5 Sensation & Perception · 6 Learning · 7 Thinking & Intelligence · 8 Memory · 9 Lifespan Development · 10 Emotion & Motivation · 11 Personality · 12 Social · 13 Industrial-Organisational · 14 Stress & Health · 15 Disorders · 16 Therapy.
openstax.org/details/books/psychology-2e

FREE VIDEO **Crash Course Psychology** (YouTube, ~40 × 10-min episodes). YouTube playlist · Crash Course channel

FREE COURSE **The Science of Well-Being** (Yale, Laurie Santos · Coursera, audit free).
coursera.org/learn/the-science-of-well-being

FREE REFERENCE **APA Dictionary of Psychology**. Look up any term. dictionary.apa.org

FREE · YOUR FUTURE SYLLABUS **IGNOU eGyanKosh**, the actual MAPC course blocks. egyankosh.ac.in

Books to buy or borrow (in order of use)

#	Book	Format tip	Weeks
1	*Maybe You Should Talk to Someone*, Lori Gottlieb	Audiobook ideal	1–8
2	*Man's Search for Meaning*, Viktor Frankl	Short; audio or print	9–11
3	*Feeling Good*, David D. Burns	Print (has exercises)	13–20
4	*The Gift of Therapy*, Irvin Yalom	Audio or print	19–24
5	*Burnout*, Emily & Amelia Nagoski	Audio	28–30
6	*Dopamine Nation*, Anna Lembke	Audio	30–32
7	*Digital Minimalism*, Cal Newport	Audio	33–35
opt.	*The Skilled Helper*, Gerard Egan (counselling textbook)	Library / used copy	22–23

A3. How to study so it sticks

You're an adult learner with a job, so use what memory research says works (more on this in Module 5).

1. **Retrieval practice beats re-reading.** After each module, close the guide and write what you remember, then check. The self-check questions at the end of every module are built for this.
2. **Space it out.** Revisit each module briefly 1 week and 1 month later. The plan builds in review weeks.
3. **Connect it to yourself.** Information linked to your own life is remembered far better (the "self-reference effect"). Every module has a "Try this" exercise for that reason.
4. **Explain it simply.** If you can explain an idea to a friend in two minutes without jargon, you've understood it.
5. **Listening counts, if you pause.** After a podcast, take 60 seconds to say out loud the one idea you'll keep.

Your learning notebook: the 3-line method

> After every study block, write:
> **1. Idea:** one thing I learned.
> **2. Mirror:** where I've seen it in my life, my work, or people around me.
> **3. Question:** something I still wonder about.
> Over 36 weeks this becomes 100+ entries: your own textbook, a source for your statement of purpose, and a list of questions to bring to your degree.

One caution before you begin

Studying psychology often stirs up your own memories and feelings. That's normal; most counsellors say training changed them. If something feels heavy, pause, talk to someone you trust, and consider seeing a counsellor yourself. Many programmes expect trainees to have had personal therapy anyway, and it's one of the best preparations there is.

Also expect "medical student syndrome", where you see every disorder in yourself and your friends. Reading about something doesn't qualify you to diagnose it, in yourself or anyone else. Hold labels lightly.

PART B

The 36-Week Plan

Week by week, with dates, from 2 November 2026 to your July 2027 admission. Tick each week off as you go.

B1. Overview

Phase	Weeks	Months	Modules	Books
1 · Foundations	1–12	Nov–Jan	1–8	Gottlieb, Frankl
2 · Person, distress, craft	13–25	Feb–Apr	9–14	Burns, Yalom
3 · Research, niche, admission	26–36	May–Jul	15–19	Nagoski, Lembke, Newport

Icons: 📖 read · 🎬 watch · 🎧 listen · 💻 online course or tool · 📚 book · ✍️ reflect/exercise · 🧪 case practice · 🗂️ admission task · 🔁 review

Crash Course episode titles change over time, so the plan names topics, not episode numbers. Open the playlist and pick the episodes on that topic (usually 1–3 per week).

Phase 1 · Foundations

Weeks 1–12 · Nov 2026 – Jan 2027 · How the mind works: brain, perception, learning, memory, thinking, emotion, development. Maps to IGNOU MPC-001 and MPC-002.

☐ **Week 1: Welcome to psychology** · Module 1 — 02 Nov – 08 Nov 2026

- 📖 Module 1 (sections 1.1–1.3) · skim OpenStax ch.1
- 🎬 Crash Course Psychology: the introductory episode
- 🎧 Start audiobook: *Maybe You Should Talk to Someone* (ch. 1–6)
- ✍️ Set up your notebook. First entry: why I want to do this.

☐ **Week 2: Lenses & the science of psychology** · Module 1 — 09 Nov – 15 Nov 2026

- 📖 Module 1 (1.4–end) · skim OpenStax ch.2 intro
- 🎬 Crash Course Psychology: the research-methods episode
- 🎧 Hidden Brain: any 2 episodes that catch your eye · Gottlieb continues

☐ **Week 3: The brain and nervous system** · Module 2 — 16 Nov – 22 Nov 2026

- 📖 Module 2 (2.1–2.3) · OpenStax ch.3
- 🎬 Crash Course Psychology: the episodes on the brain and neurons
- 🎧 Gottlieb continues
- 💻 Enrol (audit) in Science of Well-Being. Do Week 1 and take the happiness survey.

☐ **Week 4: Stress body, sleep & consciousness** · Module 2 — 23 Nov – 29 Nov 2026

- 📖 Module 2 (2.4–end) · OpenStax ch.4
- 🎬 Crash Course Psychology: the sleep / consciousness episodes
- 💻 Science of Well-Being Week 2
- ✍️ Try this: one-week sleep log

☐ **Week 5: Sensation, perception & attention** · Module 3 — 30 Nov – 06 Dec 2026

- 📖 Module 3 · OpenStax ch.5
- 🎬 Crash Course Psychology: sensation and perception episodes
- 💻 Science of Well-Being Week 3
- 🎧 Speaking of Psychology: one episode on attention or focus

☐ **Week 6: How we learn** · Module 4 — 07 Dec – 13 Dec 2026

- 📖 Module 4 · OpenStax ch.6
- 🎬 Crash Course Psychology: the learning / conditioning episodes
- 💻 Science of Well-Being Week 4
- ✍️ Try this: map one of your habits as cue → behaviour → reward

☐ **Week 7: Memory** · Module 5 — 14 Dec – 20 Dec 2026

- 📖 Module 5 · OpenStax ch.8
- 🎬 Crash Course Psychology: the memory episodes
- 💻 Science of Well-Being Week 5
- 🎧 Gottlieb continues

☐ **Week 8: Thinking, biases & intelligence** · Module 6 — 21 Dec – 27 Dec 2026

- 📖 Module 6 · OpenStax ch.7
- 🎬 Crash Course Psychology: the cognition and intelligence episodes
- 💻 Science of Well-Being Week 6
- 🎧 Finish Gottlieb

Finished your first book.

☐ **Week 9: Motivation & emotion** · Module 7 — 28 Dec – 03 Jan 2027

- 📖 Module 7 (7.1–7.3) · OpenStax ch.10
- 🎬 Crash Course Psychology: the emotion and motivation episodes
- 💻 Science of Well-Being Week 7
- 🎧 Start *Man's Search for Meaning* (Frankl)

☐ **Week 10: Stress & coping** · Module 7 — 04 Jan – 10 Jan 2027

- 📖 Module 7 (7.4–end) · OpenStax ch.14
- 🎬 Crash Course Psychology: the stress episode(s)
- 💻 Science of Well-Being Week 8
- ✍️ Try this: your personal stress-and-coping map

☐ **Week 11: Development across the lifespan** · Module 8 — 11 Jan – 17 Jan 2027

- 📖 Module 8 (8.1–8.3) · OpenStax ch.9
- 🎬 Crash Course Psychology: the developmental episodes
- 💻 Science of Well-Being Week 9
- 🎧 Finish Frankl

☐ **Week 12: Attachment & adulthood · Phase 1 review** · Module 8 — 18 Jan – 24 Jan 2027

- 📖 Module 8 (8.4–end)
- 💻 Science of Well-Being Week 10 (finish!)
- 🔁 Review: redo the self-check questions for Modules 1–8 from memory
- ✍️ Write one page: what surprised me most in Phase 1

✅ **Phase 1 complete: you now know the foundations of the first-year syllabus.**

Phase 2 · The person, distress & the craft of helping

Weeks 13–25 · Feb – Apr 2027 · Personality, social psychology, psychopathology, counselling approaches, skills, ethics. Maps to MPC-003, MPC-004 and the Year-2 counselling courses.

☐ **Week 13: Personality I: the big theories** · Module 9 — 25 Jan – 31 Jan 2027

- 📖 Module 9 (9.1–9.4) · OpenStax ch.11
- 🎬 Crash Course Psychology: the personality episodes
- 🎧 Speaking of Psychology: 2 episodes
- 📚 Start *Feeling Good* (Part I)

☐ **Week 14: Personality II: traits, assessment, Indian views** · Module 9 — 01 Feb – 07 Feb 2027

- 📖 Module 9 (9.5–end)
- 💻 Take the free IPIP Big Five test and reflect on your scores
- 📚 *Feeling Good* continues: do the mood checklist

☐ **Week 15: Social psychology I** · Module 10 — 08 Feb – 14 Feb 2027

- 📖 Module 10 (10.1–10.3) · OpenStax ch.12
- 🎬 Crash Course Psychology: the social-thinking and social-influence episodes
- 📚 *Feeling Good*: the chapter on cognitive distortions

☐ **Week 16: Social psychology II: groups, prejudice, culture** · Module 10 — 15 Feb – 21 Feb 2027

- 📖 Module 10 (10.4–end)
- 🎧 Hidden Brain: one episode on bias or belonging
- ✍️ Try this: spot the fundamental attribution error in yourself this week

☐ **Week 17: When the mind is in distress I** · Module 11 — 22 Feb – 28 Feb 2027

- 📖 Module 11 (11.1–11.3) · OpenStax ch.15
- 🎬 Crash Course Psychology: the psychological-disorders episodes
- 📚 *Feeling Good*: do 3 thought records on yourself

☐ **Week 18: Distress II: risk, referral & first aid** · Module 11 — 01 Mar – 07 Mar 2027

- 📖 Module 11 (11.4–end)
- 💻 Do the free **Psychological First Aid** course (DisasterReady, ~2 hrs)
- 📄 Skim the WHO PFA Guide for Field Workers

🏅 **PFA certificate earned.**

☐ **Week 19: Counselling approaches I: relationship-based** · Module 12 — 08 Mar – 14 Mar 2027

- 📖 Module 12 (12.1–12.3) · OpenStax ch.16
- 🎬 Crash Course Psychology: the therapy episodes
- 🎧 Start *The Gift of Therapy* (Yalom)

☐ **Week 20: Counselling approaches II: CBT & REBT in depth** · Module 12 — 15 Mar – 21 Mar 2027

- 📖 Module 12 (12.4–12.5)
- 📚 Finish the core of *Feeling Good*
- 💻 Browse beckinstitute.org: what CBT training looks like

☐ **Week 21: Counselling approaches III: newer & brief therapies** · Module 12 — 22 Mar – 28 Mar 2027

- 📖 Module 12 (12.6–end)
- 🎧 Yalom continues
- ✍️ Try this: the miracle question, on yourself

☐ **Week 22: The counselling process & micro-skills** · Module 13 — 29 Mar – 04 Apr 2027

- 📖 Module 13 (13.1–13.3)
- 🎧 Esther Perel: *Where Should We Begin?*, one episode (listen for skills, not story)
- ✍️ Practise reflective listening in 3 everyday conversations

☐ **Week 23: Case conceptualisation & practice** · Module 13 — 05 Apr – 11 Apr 2027

- 📖 Module 13 (13.4–end)
- 🧪 Part D: work through Cases 1–3
- 🗣️ Role-play Script 1 with a trusted friend (Part D)

☐ **Week 24: Ethics & Indian law** · Module 14 — 12 Apr – 18 Apr 2027

- 📖 Module 14
- 🎧 Finish Yalom
- 📄 Read the Tele-MANAS page and the MHCA 2017 summary

☐ **Week 25: Phase 2 review + admission research** — 19 Apr – 25 Apr 2027

- 🔁 Redo self-check questions for Modules 9–14
- 🗂️ **Admission:** Part E1. Check NCAHP's position on distance (ODL) degrees.
- ✍️ One page: which approach (Module 12) fits me best, and why

✅ **Phase 2 complete: you understand distress and the craft of helping.**

Phase 3 · Research, your niche & admission

Weeks 26–36 · May – Jul 2027 · Research and statistics, workplace and digital wellbeing, the Indian context, self-care, and admission. Maps to MPC-005 and MPC-006.

☐ **Week 26: Research methods** · Module 15 — 26 Apr – 02 May 2027

- 📖 Module 15 (15.1–15.3) · OpenStax ch.2
- 🎬 Crash Course Psychology: research-methods episode (rewatch)
- 🎧 Speaking of Psychology: one episode, and ask 'how did they know that?'

☐ **Week 27: Statistics for psychology** · Module 15 — 03 May – 09 May 2027

- 📖 Module 15 (15.4–end)
- 💻 Khan Academy: descriptive statistics + normal distribution units
- 🗂️ Admission: look for the IGNOU July 2027 notification and prospectus

☐ **Week 28: Workplace wellbeing I: burnout & stress at work** · Module 16 — 10 May – 16 May 2027

- 📖 Module 16 (16.1–16.3) · OpenStax ch.13
- 🎧 WorkLife with Adam Grant: 2 episodes
- 🎧 Start *Burnout* (Nagoski)

☐ **Week 29: Workplace wellbeing II: careers, teams & the counsellor's role** · Module 16 — 17 May – 23 May 2027

- 📖 Module 16 (16.4–end)
- 🎧 How's Work? with Esther Perel: 2 episodes
- 🧪 Part D: Cases 4–5 (workplace)

☐ **Week 30: Digital-age wellbeing I: the attention machine** · Module 17 — 24 May – 30 May 2027

- 📖 Module 17 (17.1–17.3)
- 🎧 Your Undivided Attention: 2 episodes
- 🎧 Finish *Burnout*, start *Dopamine Nation*
- ✍️ Try this: your 7-day digital audit

☐ **Week 31: Digital-age wellbeing II: loneliness, AI & interventions** · Module 17 — 31 May – 06 Jun 2027

- 📖 Module 17 (17.4–end)
- 🎧 *Dopamine Nation* continues
- 🧪 Part D: Cases 6–7 (digital)

☐ **Week 32: Mental health in India** · Module 18 — 07 Jun – 13 Jun 2027

- 📖 Module 18
- 🎧 Finish *Dopamine Nation*
- 🗂️ **Admission: apply** when the IGNOU July window opens (Part E2 step by step)

📨 **Application submitted (or ready to submit).**

☐ **Week 33: Looking after the helper** · Module 19 — 14 Jun – 20 Jun 2027

- 📖 Module 19
- 🎧 Start *Digital Minimalism*
- ✍️ Draft your personal self-care plan (template in Module 19)

☐ **Week 34: Integration** — 21 Jun – 27 Jun 2027

- 🧪 Part D: Case 8 (integrated) + re-do Case 1 and compare with Week 23
- ✍️ Write your statement of purpose (Part E4), even if IGNOU doesn't need one
- 🎧 *Digital Minimalism* continues

☐ **Week 35: Final review** — 28 Jun – 04 Jul 2027

- 🔁 Final self-check across all 19 modules, and mark weak spots
- 📄 Preview MPC-001 blocks on eGyanKosh
- 🎧 Finish *Digital Minimalism*

☐ **Week 36: Launch week** — 05 Jul – 11 Jul 2027

- 🗂️ Confirm enrolment, study centre and practical schedule
- 📅 Plan your Year-1 weekly routine (Part E5)
- ✍️ Letter to yourself, to open after the degree: why I started

🎉 **Ready for the MA. You're starting ahead of most of your batch.**

PART C

Study Modules

Nineteen modules, written for this guide. Each one has: why it matters for a counsellor · the key ideas · a link to your UX experience · a "Try this" exercise · self-check questions (answers in Part F) · sources.

MODULE 1 · PHASE 1 · WEEKS 1–2

What Psychology Is

IGNOU link: background to all MPC courses · OpenStax ch. 1–2

Why it matters: before you can help a mind, you need a map of how psychologists think about minds, and how they tell knowledge apart from opinion.

1.1 A working definition

Psychology is the scientific study of mind and behaviour. "Mind" covers what can't be seen directly: thoughts, feelings, memories, motives. "Behaviour" is what can be observed: actions, speech, even physiological responses. "Scientific" is the key word. Psychology tests ideas against evidence instead of relying on intuition, authority or anecdote.

Psychology overlaps with, but differs from:

- **Psychiatry:** a medical specialty. Psychiatrists are doctors who diagnose and can prescribe medication.
- **Counselling:** applied helping. It uses psychological knowledge to help people cope, decide and grow. Counselling psychology tends to focus on everyday distress, adjustment and wellbeing, more than severe mental illness.
- **Clinical psychology:** assessment and treatment of more serious psychological disorders, often in hospitals. In India this is RCI-licensed.
- **Pop psychology / self-help:** sometimes useful, often oversimplified, and rarely tested. You'll learn to tell the difference.

1.2 A very short history

When	Who / what	Big idea
Ancient	Greek philosophers; Indian traditions (Upanishads, Buddhism, Yoga, Sankhya)	Questions about mind, self, suffering and consciousness long predate psychology as a science. Indian thought especially developed detailed models of mind and of practices like meditation.
1879	Wilhelm Wundt, Leipzig	First psychology laboratory, usually counted as the birth of scientific psychology. Studied conscious experience through trained introspection (**structuralism**).
1890	William James	**Functionalism**: what is the mind *for*? How does it help us adapt?
~1900	Sigmund Freud	**Psychoanalysis**: unconscious motives and early experience shape us. The "talking cure".
1913–1950s	Watson, Skinner	**Behaviourism**: study only observable behaviour, learned through conditioning.
1950s–60s	Rogers, Maslow	**Humanistic psychology**: people are growth-oriented; free will, meaning, self-actualisation.
1960s–	Neisser, Beck, Ellis	**Cognitive revolution**: mind as information processor. Leads to cognitive therapies.
1990s–	Neuroscience, positive psychology, cultural psychology	Brain imaging; Seligman's positive psychology (1998); growing recognition that Western findings don't always generalise.
India	N.N. Sengupta (Calcutta University, 1916, first psychology department); Girindrasekhar Bose (Indian Psychoanalytical Society, 1922)	A long Indian tradition of psychology, now including indigenous approaches (e.g. Triguna personality model, yoga-based interventions).

1.3 Branches you'll meet

Cognitive · Developmental · Social · Personality · Biological / Neuro · Clinical · **Counselling** · Health · **Industrial-Organisational (I-O)** · Educational · Forensic · Positive · Cross-cultural. The two in bold are your future home. The rest are the foundation.

1.4 The six lenses (perspectives)

The same problem, say a young engineer who can't sleep before Monday, looks different through each lens:

Lens	Core question	Applied to the engineer
Biological	What's happening in the body and brain?	Stress hormones, caffeine, irregular sleep schedule, genetic anxiety tendency
Behavioural	What has been learned through consequences?	Checking email late at night is reinforced by brief relief, so the habit grows
Cognitive	What are they thinking or believing?	"If the release fails, I'll be fired." Catastrophic predictions
Psychodynamic	What unconscious patterns or past relationships are active?	A strict, critical parent; the manager now "feels like" that parent
Humanistic	Is the person able to grow and live by their values?	Job conflicts with what they really care about; little sense of meaning
Socio-cultural	What do family, culture and society add?	Family's sacrifice for their education; the EMI; fear of letting everyone down

The modern integrative view is the **biopsychosocial model** (George Engel, 1977): problems almost always have biological, psychological and social threads at once. Good counsellors hold all three.

1.5 How psychology knows things

Psychology uses the **scientific method**: observe → form a testable hypothesis → collect data systematically → analyse → publish → others try to replicate. Key safeguards: careful measurement, control groups, random assignment, peer review, and **replication**. Since about 2011 the field has been through a "replication crisis", in which some famous findings didn't hold up when repeated. This is healthy self-correction, and a reason to prefer findings backed by many studies over one striking experiment. (Module 15 goes deeper.)

Critical-thinking checklist for any psychology claim you hear on a podcast:

1. What's the evidence: one study, or many?
2. Who was studied? (US college students ≠ Indian working adults)
3. Is it correlation or causation?
4. How big is the effect, not just "is it significant"?
5. Who benefits if I believe it?

> **Your UX edge:** You already think like a researcher: hypotheses, usability tests, avoiding leading questions, small-sample caution, triangulating qualitative and quantitative data. Psychology's research methods will feel familiar, and much of HCI is built on cognitive psychology.

Try this

Pick one of your own recurring struggles (e.g. procrastination). Write one sentence about it for each of the six lenses. Notice how each lens suggests a different kind of help.

Self-check

1. How does counselling psychology differ from clinical psychology and psychiatry?
2. Who set up the first psychology lab, and when?
3. Name the six perspectives and give the core question of each.
4. What does the biopsychosocial model claim?
5. Why does replication matter?

Go deeper: OpenStax Psychology 2e, ch. 1–2 · Crash Course Psychology, intro episode · Biopsychosocial model · Replication crisis · Psychology in India

MODULE 2 · PHASE 1 · WEEKS 3–4

The Biological Basis of Behaviour

OpenStax ch. 3–4 · Background for MPC-001

Why it matters: you won't prescribe, but you must understand the body under stress, why sleep matters so much, and when a problem might need a doctor.

2.1 Neurons and neurotransmitters

The brain has roughly 86 billion **neurons**. A neuron receives signals through its dendrites, fires an electrical impulse (the **action potential**) down its axon, and releases chemical messengers, called **neurotransmitters**, across a tiny gap (the **synapse**) to the next neuron.

Neurotransmitter	Main roles (simplified)	Why a counsellor hears about it
Serotonin	Mood regulation, sleep, appetite	Many antidepressants (SSRIs) act on serotonin
Dopamine	Reward *prediction*, motivation, learning, movement	Addiction, habit-forming apps, ADHD, psychosis
Norepinephrine	Alertness, arousal, fight-or-flight	Anxiety, panic
GABA	Main "brake": inhibits neural firing	Anti-anxiety drugs (benzodiazepines), alcohol
Glutamate	Main "accelerator": learning and memory	Memory formation
Endorphins	Natural pain relief, pleasure	Exercise "high"

> **Caution:** "Depression = low serotonin" is an oversimplification. Research doesn't support a simple chemical-imbalance story. Brain chemistry is one thread in the biopsychosocial picture, not the whole cause.

2.2 The nervous system

- **Central nervous system (CNS):** brain and spinal cord.
- **Peripheral nervous system:**
  - **Somatic:** voluntary movement and senses.
  - **Autonomic (ANS):** automatic body functions, with two branches:
    - **Sympathetic:** "fight or flight". Heart races, breathing speeds up, digestion slows.
    - **Parasympathetic:** "rest and digest". Calms the body back down.

Much of anxiety is the sympathetic system firing when there's no real physical danger. Slow breathing with long exhales is one of the few ways to deliberately activate the parasympathetic branch, which is why breathing exercises appear in almost every anxiety toolkit.

2.3 Brain regions worth knowing

Region	Key functions
Prefrontal cortex (front of frontal lobe)	Planning, decision-making, impulse control, regulating emotion. Matures until the mid-20s.
Amygdala	Detects threat and emotional significance; triggers fear responses fast
Hippocampus	Forming new long-term memories; context. Can be affected by chronic stress.
Hypothalamus	Hunger, thirst, temperature, and starts the hormonal stress response
Thalamus	Relay station for sensory information
Cerebellum	Coordination, balance, motor learning
Brainstem	Breathing, heart rate, sleep-wake cycles
Lobes: occipital / temporal / parietal / frontal	Vision / hearing, language and memory / touch and spatial / planning, movement, personality

A useful counselling metaphor: under strong emotion the fast "alarm" (amygdala) can overpower the slower "thinking brain" (prefrontal cortex). It's hard to reason with someone mid-panic, so calm the body first, then think.

Neuroplasticity: the brain keeps changing its connections with experience throughout life. This is the biological basis for hope in therapy: practised new patterns of thought and behaviour physically reshape neural pathways.

2.4 The stress response (HPA axis)

When you perceive a threat: **Hypothalamus → Pituitary gland → Adrenal glands** release **cortisol** (and adrenaline through the sympathetic system). In the short term this is useful: energy, focus, readiness. When it stays switched on for weeks or months (chronic stress), it's linked to poor sleep, weaker immunity, digestive problems, memory and concentration difficulties, and higher risk of anxiety and depression. Module 7 picks this up.

2.5 Sleep and consciousness

- Sleep runs in ~90-minute cycles: **NREM stages 1–3** (stage 3 = deep, restorative sleep) and **REM** (vivid dreams, emotional processing, memory consolidation). REM periods get longer towards morning.
- Adults generally need about **7–9 hours**. Our internal clock (**circadian rhythm**) is set mainly by light.
- Poor sleep and mental health feed each other: insomnia is both a symptom and a risk factor for depression and anxiety.
- **Sleep hygiene** basics: consistent wake time, morning light, no caffeine after early afternoon, cool dark room, screens and work off before bed, get up if you can't sleep after ~20 minutes. For chronic insomnia, the first-line treatment is **CBT for Insomnia (CBT-I)**, not sleeping pills.
- Other states of consciousness: meditation, hypnosis, and drug effects (depressants like alcohol, stimulants like caffeine and nicotine, hallucinogens).

2.6 When to think "medical first"

Some psychological symptoms have physical causes, such as thyroid problems (low mood or anxiety), anaemia or vitamin B12/D deficiency (fatigue), medication side-effects, or substance use. Sudden,

unusual changes, especially with physical symptoms, should go to a doctor before or alongside counselling. "Rule out the medical" is a basic professional habit.

Try this: one-week sleep log

Each morning note bedtime, wake time, rough sleep quality (1–10), caffeine after 2 pm (yes/no), and screen use in bed (yes/no). At week's end, look for patterns. You've just done a basic behavioural assessment, something you'll do with clients.

Self-check

1. What are the two branches of the autonomic nervous system, and what does each do?
2. Which brain region acts as a threat detector? Which one regulates impulses?
3. Describe the HPA axis in one sentence.
4. Why is "depression = low serotonin" misleading?
5. What is the first-line treatment for chronic insomnia?

Go deeper: OpenStax Psychology 2e, ch. 3–4 · HPA axis · Neuroplasticity · CBT-I

MODULE 3 · PHASE 1 · WEEK 5

Sensation, Perception & Attention

OpenStax ch. 5 · MPC-001 (cognitive processes)

Why it matters: people react not to reality but to their perception of it. Attention is also the main currency of the digital world (Module 17).

3.1 Sensation vs perception

- **Sensation:** sense organs detect raw energy (light, sound waves, pressure) and convert it to neural signals (**transduction**).
- **Perception:** the brain organises and interprets those signals into meaningful experience.
- **Bottom-up processing:** built up from the raw data. **Top-down processing:** shaped by expectations, knowledge and mood. An anxious person scanning a room "sees" disapproval more readily, which is top-down perception in action.
- **Thresholds:** the *absolute threshold* is the minimum detectable stimulus. The *just-noticeable difference* follows **Weber's law**: the change needed is a constant proportion of the original. **Sensory adaptation:** we stop noticing constant stimuli.

3.2 Gestalt principles

"The whole is different from the sum of its parts." The brain automatically groups by **proximity, similarity, closure, continuity, figure–ground** and **common fate**. You already use these in layouts. In counselling, a similar tendency appears at the level of meaning: people "close the gaps" in ambiguous social situations, often with their fears.

3.3 Attention

Concept	What it means
Selective attention	Focusing on one stream while filtering others. The **cocktail-party effect**: you still notice your name across the room.
Divided attention	"Multitasking" is mostly rapid switching, with a cost in errors and time.
Inattentional blindness	Missing obvious things when focused elsewhere (Simons & Chabris's 1999 "invisible gorilla" study).
Change blindness	Failing to notice changes between views.
Cognitive load	Working memory is limited. Overloading it hurts learning and decisions (Sweller).
Attentional bias	Anxiety pulls attention towards threat cues; depression lingers on negative information.

Early theories: Broadbent's **filter model** (early selection), Treisman's **attenuation model** (unattended input is turned down, not blocked), and Kahneman's **capacity model** (attention as limited mental resources).

> **Your UX edge:** Gestalt, cognitive load, Fitts's and Hick's laws, banner blindness: you've used perception and attention research in your work. In counselling, the same science explains why a stressed client can't "just focus", and why a cluttered life overloads attention just like a cluttered interface.

Try this

For one day, notice when your attention gets "pulled" by notifications. Count them. Then notice one moment when your mood changed how you *perceived* something neutral (a message, a face, a tone).

Self-check

1. Distinguish sensation from perception, and bottom-up from top-down processing.
2. Name four Gestalt principles.
3. What is inattentional blindness? Give the classic study.
4. How does anxiety bias attention?

Go deeper: OpenStax Psychology 2e, ch. 5 · Inattentional blindness · Attentional bias

MODULE 4 · PHASE 1 · WEEK 6

Learning

OpenStax ch. 6 · MPC-001 (learning)

Why it matters: fears, habits and many problem behaviours are learned, and what's learned can be unlearned. Behaviour therapy and much of the digital-wellbeing work rest on this module.

4.1 Classical conditioning (Pavlov)

A neutral stimulus becomes paired with one that naturally triggers a response, until the neutral one alone triggers it.

Term	Pavlov's dogs	Human example
Unconditioned stimulus (UCS)	Food	A harsh public scolding
Unconditioned response (UCR)	Salivation	Fear, shame
Conditioned stimulus (CS)	Bell	The manager's calendar invite titled "Quick chat"
Conditioned response (CR)	Salivation to bell	Dread when any "Quick chat" invite appears

Key processes: **acquisition**, **extinction** (CS without UCS weakens the response), **spontaneous recovery**, **generalisation** (similar stimuli trigger it too), **discrimination**. Watson's "Little Albert" study (1920) showed that fear could be conditioned in a child. It's ethically unacceptable today, but historically important for understanding phobias.

4.2 Operant conditioning (Thorndike, Skinner)

Behaviour is shaped by its consequences.

	Add something	Remove something
Increase behaviour	Positive reinforcement (praise, likes)	Negative reinforcement (relief: avoiding a scary meeting removes anxiety)
Decrease behaviour	Positive punishment (scolding)	Negative punishment (losing privileges)

Negative reinforcement is not punishment. It's the removal of something unpleasant, and it's the engine of **avoidance**: avoiding the feared thing brings instant relief, so avoidance grows, and the fear never gets disproven. That's why most anxiety treatment involves gradual **exposure**.

Schedules of reinforcement

Schedule	Rule	Example	Pattern
Fixed ratio	Every n responses	Piece-rate pay	High rate, pause after reward
Variable ratio	After an unpredictable number	Slot machines, pull-to-refresh, loot boxes	**Highest, steadiest rate; most resistant to extinction**
Fixed interval	First response after a set time	Monthly salary, checking just before a deadline	Scalloped (bursts near time)
Variable interval	First response after unpredictable time	Checking email or messages	Slow, steady checking

Other concepts: **shaping** (reinforcing successive approximations), **extinction burst** (behaviour briefly gets *worse* when reinforcement stops; warn clients about this!), **Premack principle** (use a preferred activity to reward a less preferred one).

4.3 Observational learning (Bandura)

We learn by watching others, as Bandura's Bobo doll studies (1961) showed. Four steps: **attention → retention → reproduction → motivation**. This underlies modelling in therapy and the influence of social media role models. Bandura later developed **self-efficacy** (Module 9).

4.4 Learned helplessness (Seligman)

When individuals repeatedly experience uncontrollable bad events, they may stop trying even when control becomes possible. This contributed to models of depression. Seligman's later work emphasised **explanatory style**: pessimists explain bad events as *permanent, pervasive, personal* ("It's me, it's everything, it'll never change"). Learning a more balanced style is part of "learned optimism".

4.5 Cognitive learning

Not all learning is stimulus–response: Tolman's rats formed **cognitive maps**, and Köhler's chimps showed **insight learning**. Learning also involves expectations and understanding, which bridges to cognitive therapy.

> **Your UX edge:** Streaks, badges, notifications and infinite feeds are operant conditioning, often on variable-ratio schedules. You've seen how products are built to form habits. As a counsellor you can explain this to clients without shame ("this was designed to hook you"), then redesign their cues and rewards with them.

Try this: habit map

Pick one habit you'd like to change. Write: **cue** (what triggers it) → **behaviour** → **reward** (what you get, including relief) → **schedule** (predictable or variable?). Then design one change to the cue and one alternative behaviour that gives a similar reward.

Self-check

1. In classical conditioning, what's the difference between the UCS and the CS?
2. Explain negative reinforcement, and how it maintains avoidance.
3. Which reinforcement schedule is most resistant to extinction? Give a digital example.
4. What is an extinction burst, and why warn clients about it?
5. What are Bandura's four steps of observational learning?

Go deeper: OpenStax Psychology 2e, ch. 6 · Reinforcement & schedules · Learned helplessness · Bobo doll experiment

MODULE 5 · PHASE 1 · WEEK 7

Memory

OpenStax ch. 8 · MPC-001 (memory)

Why it matters: clients tell you their story from memory, and memory is reconstructive, emotional and selective. It also helps you study better.

5.1 Three stages

Encoding (getting information in) → **storage** (keeping it) → **retrieval** (getting it back out). Forgetting can happen at any stage.

5.2 The multi-store model (Atkinson & Shiffrin, 1968)

Store	Capacity	Duration
Sensory memory	Large	< 1 sec (iconic, visual) to ~2–4 sec (echoic, auditory)
Short-term memory	Limited: Miller's "7 ± 2" (1956); later work suggests ~4 chunks	~15–30 sec without rehearsal
Long-term memory	Effectively unlimited	Minutes to a lifetime

Chunking (grouping items) expands short-term capacity, which you already use when designing phone-number fields and menus.

5.3 Working memory (Baddeley & Hitch, 1974)

Short-term memory is better seen as an active workspace with parts: the **central executive** (directs attention), the **phonological loop** (inner voice), the **visuospatial sketchpad** (mental images), and the **episodic buffer** (added in 2000, integrates information). Anxiety and rumination eat working-memory capacity, which is one reason anxious people find it hard to concentrate.

5.4 Levels of processing (Craik & Lockhart, 1972)

Memory depends on *how deeply* information is processed, not just on which "store" it's in. **Shallow** processing (how a word looks) < **phonemic** (how it sounds) < **semantic** (what it means). Deep, meaningful, self-related processing produces durable memories. *(This model is a frequent IGNOU exam question.)*

5.5 Types of long-term memory

- **Explicit (declarative):** consciously recalled.
  - **Episodic:** personal events ("my first day at work").
  - **Semantic:** facts and knowledge ("Delhi is the capital").
- **Implicit (non-declarative):** shows up in behaviour without conscious recall. **Procedural** skills (riding a bike), **priming**, and classically conditioned emotional responses.

5.6 Forgetting

- **Ebbinghaus's forgetting curve:** rapid loss soon after learning, then levelling off. Spaced review flattens it.
- **Interference:** *proactive* (old learning disrupts new) and *retroactive* (new learning disrupts old).

- **Retrieval failure:** it's stored but you can't access it. **Encoding specificity** (Tulving): retrieval is easiest when the context or state matches learning. Mood-congruent memory means a sad mood brings sad memories to mind more easily, which feeds depression.
- **Motivated forgetting / repression:** a Freudian idea. Modern research is cautious about "recovered memories".

5.7 Memory is reconstructive

We rebuild memories each time we recall them, filling gaps with expectations (Bartlett's "schemas", 1932). Loftus & Palmer (1974) showed that the wording of a question ("smashed" vs "hit") changed people's estimates of car speed and even created false memories of broken glass. **The misinformation effect** means leading questions can change memories.

> **For counsellors:** (1) Ask open, non-leading questions, the same discipline as unbiased user interviews. (2) Be careful with "suggesting" past events, especially trauma. (3) A client's memory is *their truth about their experience*. Work with its meaning; you're not a fact-checker.

5.8 Memory techniques (for you and your clients)

Retrieval practice (testing yourself) · **spacing** · **elaboration** (connect to what you know) · **dual coding** (words + visuals) · **mnemonics** (method of loci, acronyms like OCEAN) · **sleep** (consolidation happens during sleep).

Try this
Without looking back, write everything you remember from Module 4. Then check. What did you forget? That's the forgetting curve. Do it again next week and notice the improvement.

Self-check

1. Name the components of Baddeley & Hitch's working memory model.
2. Explain the levels-of-processing model with an example.
3. Distinguish episodic, semantic and procedural memory.
4. What did Loftus & Palmer show, and why does it matter in counselling?
5. What is mood-congruent memory?

Go deeper: OpenStax Psychology 2e, ch. 8 · Levels of processing · Baddeley's model · Misinformation effect

MODULE 6 · PHASE 1 · WEEK 8

Thinking, Decisions & Intelligence

OpenStax ch. 7 · MPC-001 (thinking, problem solving, intelligence, creativity)

Why it matters: thinking errors are what cognitive therapy works on, and decisions are often what clients bring ("Should I quit?").

6.1 Concepts, schemas and problem solving

- We organise knowledge into **concepts** and **prototypes** (the "best example" of a category), and into **schemas**, mental frameworks that guide expectations. In therapy, **core beliefs** ("I'm unlovable") act like deep schemas.
- **Problem-solving strategies:** trial and error, **algorithms** (step-by-step, guaranteed but slow), **heuristics** (mental shortcuts, fast but error-prone), and insight.
- **Obstacles: functional fixedness** (seeing objects only in their usual use), **mental set** (sticking with old solutions), and **confirmation bias**.

6.2 Two systems (Kahneman)

System 1	System 2
Fast, automatic, intuitive, emotional, effortless	Slow, deliberate, logical, effortful
"That message sounded angry!"	"Let me reread it. Was it actually angry?"

Much of CBT trains people to catch System 1 conclusions and check them with System 2.

6.3 Heuristics and biases (Tversky & Kahneman, 1974 onwards)

Bias	What it is	Counselling example
Availability	Judging likelihood by how easily examples come to mind	After news of layoffs, "I'm definitely next"
Representativeness	Judging by resemblance to a stereotype, ignoring base rates	"Successful founders drop out, so I should too"
Anchoring	Over-relying on the first number or idea	A first salary anchors all later self-worth
Confirmation bias	Seeking evidence that fits what we believe	Noticing only proof that "nobody likes me"
Loss aversion	Losses loom larger than equal gains	Staying in a bad job to avoid "losing" stability
Sunk-cost fallacy	Continuing because of past investment	"I've spent 5 years on UX, so I can't switch"
Framing	Same facts, different choice depending on wording	"90% survive" vs "10% die"
Hindsight bias	"I knew it all along"	Harsh self-blame after a bad outcome

6.4 Intelligence: main theories

Theory	Key claim
Spearman (1904): **g factor**	A general intelligence underlies performance across tasks, plus specific abilities (s)
Thurstone: primary mental abilities	Seven separate abilities (verbal comprehension, word fluency, number, space, memory, perceptual speed, reasoning)
Cattell (and Horn): fluid vs crystallised	**Fluid** = novel reasoning (peaks early adulthood); **crystallised** = accumulated knowledge (grows with age)
Guilford: **structure of intellect**	Intelligence as combinations of operations × contents × products (up to 150+ abilities); introduced **convergent** vs **divergent** thinking
Gardner (1983): multiple intelligences	Linguistic, logical-mathematical, spatial, musical, bodily-kinaesthetic, interpersonal, intrapersonal, naturalist. Popular in education, but weak empirical support.
Sternberg: triarchic theory	**Analytical, creative, practical** intelligence
Emotional intelligence (Salovey & Mayer; popularised by Goleman)	Perceiving, using, understanding and managing emotions
CHC model (Cattell–Horn–Carroll)	Today's dominant hierarchical model: g at the top, broad abilities below, narrow abilities at the base

Measurement: Binet (France, 1905) → Stanford-Binet; Wechsler scales (WAIS for adults, WISC for children). IQ scores are standardised to a **mean of 100, SD 15**. In India, adaptations include Bhatia's Battery of Performance Tests and Indian versions of Wechsler and Raven's Progressive Matrices. Tests must be culturally fair and validated for the population.

6.5 Creativity

Creative thinking relies on **divergent thinking** (many possible answers) as well as convergent thinking. Wallas's stages: **preparation → incubation → illumination → verification**. You'll recognise this from design ideation. Creativity relates to intelligence only up to a point.

> **Your UX edge:** You design for System 1 every day (defaults, anchoring in pricing pages, framing). That same knowledge lets you help clients notice when *their own* mind is using shortcuts against them.

Try this

Recall a recent decision that went badly. Which two biases from the table might have played a role? Rewrite the decision process as if System 2 had been in charge.

Self-check

1. Distinguish algorithms and heuristics. Give one example of each.
2. Explain availability and confirmation bias with counselling examples.
3. What's the difference between fluid and crystallised intelligence?
4. What did Guilford contribute? What is divergent thinking?
5. Name Wallas's four stages of creativity.

Go deeper: OpenStax Psychology 2e, ch. 7 · Heuristics · List of cognitive biases · CHC theory · Book: Kahneman, *Thinking, Fast and Slow*

MODULE 7 · PHASE 1 · WEEKS 9–10

Motivation, Emotion, Stress & Coping

OpenStax ch. 10 & 14 · MPC-001 / health psychology background

Why it matters: almost every client arrives with a problem of motivation ("I can't get started"), emotion ("I can't stop feeling this") or stress ("I'm overwhelmed").

7.1 Theories of motivation

Theory	Idea
Instinct	Behaviour driven by innate patterns (largely abandoned for humans)
Drive reduction (Hull)	Biological needs create drives (hunger) that we act to reduce, maintaining **homeostasis**
Arousal theory / **Yerkes–Dodson law**	Performance is best at *moderate* arousal; too little = boredom, too much = anxiety. The optimum is lower for complex tasks.
Maslow's hierarchy (1943)	Physiological → safety → love/belonging → esteem → self-actualisation. Intuitive, but the strict order isn't supported. People pursue meaning even when basic needs are unmet (see Frankl).
Self-Determination Theory (Deci & Ryan)	Three basic psychological needs: **autonomy, competence, relatedness**. When met, people show intrinsic motivation and wellbeing. Strong research support; very useful at work.
Intrinsic vs extrinsic	Doing something for its own sake vs for rewards. The **overjustification effect**: external rewards can undermine intrinsic interest.
Goal-setting (Locke & Latham)	Specific, challenging goals with feedback improve performance

7.2 What emotions are

Emotions have three components: **physiological arousal**, **expressive behaviour** (face, posture), and **conscious experience**. They're signals that carry information ("anger: a boundary was crossed"; "anxiety: something matters and feels uncertain"). Ekman proposed a set of **basic emotions** (happiness, sadness, fear, anger, surprise, disgust) with recognisable facial expressions across cultures, though cultures differ in **display rules**.

7.3 Theories of emotion

Theory	Sequence
James–Lange	Event → bodily arousal → emotion ("I feel afraid *because* I tremble")
Cannon–Bard	Event → arousal and emotion at the same time, independently
Schachter–Singer two-factor	Event → arousal + **cognitive label** → emotion. The same racing heart can be "excitement" or "fear" depending on interpretation.
Lazarus: cognitive appraisal	Event → appraisal (what does this mean for me?) → emotion. The foundation of cognitive therapies.

Emotion regulation (Gross): strategies include situation selection, attentional deployment, **cognitive reappraisal** (generally healthy), and **suppression** (often costly when habitual). Simply naming an emotion ("affect labelling") can reduce its intensity.

7.4 Stress

- **Selye's General Adaptation Syndrome: alarm → resistance → exhaustion** under prolonged stress.
- **Holmes & Rahe (1967) Social Readjustment Rating Scale:** life changes, even positive ones (marriage, a new job, moving cities), add up to stress.
- **Daily hassles** (traffic, deadlines) can predict wellbeing as well as major events.
- **Lazarus & Folkman's transactional model (1984):** stress arises from **primary appraisal** ("Is this a threat, harm or challenge?") and **secondary appraisal** ("Do I have the resources to cope?"). Stress = demands perceived to exceed resources.
- **Eustress vs distress:** some stress energises; the dose and the sense of control matter.

7.5 Coping

Type	Examples	Best when…
Problem-focused	Planning, asking for help, negotiating workload	The situation can be changed
Emotion-focused	Reappraisal, acceptance, exercise, talking it out, prayer, humour	The situation can't be changed (illness, loss)
Avoidant / maladaptive	Denial, alcohol, endless scrolling, overworking	Short-term relief, long-term cost

Protective factors: **social support** (one of the strongest buffers), sense of control, optimism, physical activity, sleep, meaning and spirituality. **Resilience** isn't a rare trait. Masten calls it "ordinary magic": common human adaptive systems working well.

Type A behaviour (competitive, hurried, hostile): the *hostility* component, not the hurry, is most linked to heart disease.

Try this: stress-and-coping map
List your top 5 current stressors. For each: (a) controllable or not? (b) what coping do I use now? (c) is it problem-focused, emotion-focused or avoidant? (d) one better-matched strategy. This is a real counselling worksheet.

Self-check

1. State the Yerkes–Dodson law.
2. What are the three needs in Self-Determination Theory?
3. Compare James–Lange, Cannon–Bard and Schachter–Singer.
4. Explain primary and secondary appraisal.
5. When is problem-focused coping better than emotion-focused coping?

Go deeper: OpenStax Psychology 2e, ch. 10 & 14 · Self-determination theory · Yerkes–Dodson law · Coping · Frankl, *Man's Search for Meaning*

MODULE 8 · PHASE 1 · WEEKS 11–12

Development Across the Lifespan

OpenStax ch. 9 · **IGNOU MPC-002: Life Span Psychology**

Why it matters: every client is somewhere on a developmental path, and much adult distress has roots in earlier stages, especially attachment.

8.1 Big debates

Nature vs nurture (today: always both, interacting; *epigenetics* shows experience changes how genes are expressed) · **continuity vs stages** · **stability vs change** · **critical vs sensitive periods**.

8.2 Cognitive development

Piaget's stage	Age (approx.)	Key achievements
Sensorimotor	0–2	Learning through senses and action; **object permanence**
Preoperational	2–7	Language, symbolic play; **egocentrism**; lacks **conservation**
Concrete operational	7–11	Logical thinking about concrete things; conservation
Formal operational	11+	Abstract, hypothetical reasoning

Piaget: children actively build knowledge through **schemas**, **assimilation** (fitting new info into existing schemas) and **accommodation** (changing schemas). **Vygotsky** stressed social and cultural learning: the **zone of proximal development** (what a learner can do with help) and **scaffolding**. That's a good model for counselling too: support, then gradually step back.

8.3 Erikson's psychosocial stages

Age	Crisis	Virtue if resolved
Infancy	Trust vs mistrust	Hope
Early childhood	Autonomy vs shame & doubt	Will
Preschool	Initiative vs guilt	Purpose
School age	Industry vs inferiority	Competence
Adolescence	**Identity vs role confusion**	Fidelity
Young adulthood	**Intimacy vs isolation**	Love
Middle adulthood	**Generativity vs stagnation**	Care
Late adulthood	Integrity vs despair	Wisdom

Your future workplace clients will mostly be wrestling with identity, intimacy and generativity: "Who am I outside my job?", "Am I connected?", "Does my work matter?" (Your own career change is a generativity question.)

Marcia's identity statuses: diffusion, foreclosure (committed without exploring, e.g. a career chosen by parents, common in India), moratorium (exploring), achievement.

8.4 Attachment

- **Bowlby:** infants are biologically driven to stay close to caregivers. Early relationships form **internal working models** of self and others.

- **Ainsworth's Strange Situation** identified patterns: **secure**, **anxious-avoidant**, **anxious-resistant/ambivalent**. Main & Solomon later added **disorganised**.
- **Adult attachment** (Hazan & Shaver, 1987): similar patterns appear in adult romantic relationships, often described on two dimensions, *anxiety* (fear of abandonment) and *avoidance* (discomfort with closeness).
- Attachment isn't destiny: "earned security" can develop through later relationships, **including the therapeutic relationship**.

8.5 Parenting, moral development and adolescence

- **Baumrind's parenting styles** (+ Maccoby & Martin): **authoritative** (warm + firm; best outcomes in most research), **authoritarian** (strict, low warmth), **permissive** (warm, few limits), **neglectful**. Cultural note: in collectivist settings like India, strictness may be experienced as care, so outcomes depend on context and warmth.
- **Kohlberg's moral development:** pre-conventional (avoid punishment, self-interest) → conventional (approval, law and order) → post-conventional (social contract, universal principles). Gilligan critiqued it for neglecting an *ethic of care*.
- **Adolescence:** puberty, peer influence, risk-taking (the reward system develops faster than the prefrontal "brakes"), identity formation. Many mental health conditions first appear between adolescence and the mid-20s.

8.6 Adulthood and ageing

- **Emerging adulthood** (Arnett): roughly 18–25/29. Exploration, instability, "feeling in-between". In India it's shaped by family expectations, marriage pressure and career choices.
- **Levinson's seasons of life** and the idea of transitions (e.g. the "age-30 transition"). The "midlife crisis" is less universal than popular culture suggests.
- **Ageing:** crystallised knowledge grows; many people report *higher* emotional wellbeing in later life (**socioemotional selectivity theory**, Carstensen: as time feels shorter, people focus on meaningful relationships).
- **Death and grief:** Kübler-Ross's five stages (denial, anger, bargaining, depression, acceptance) are popular but not a fixed sequence. Modern models (e.g. the dual-process model, continuing bonds) see grief as oscillating, individual and culturally shaped.

Try this: lifeline

Draw a line from birth to today. Mark 8–10 key events above (positive) or below (difficult) the line. Next to each, note which Erikson stage it touched. Lifelines are a real counselling tool; doing yours first teaches you how it feels.

Self-check

1. Name Piaget's four stages with one achievement each.
2. What is Vygotsky's zone of proximal development?
3. List Erikson's stages from adolescence onward.
4. Describe the four infant attachment patterns.
5. Why is Kübler-Ross's model now seen as limited?

Go deeper: OpenStax Psychology 2e, ch. 9 · Erikson's stages · Attachment theory · Emerging adulthood · IGNOU MPC-002 assignment: MPC-002 (2026–27)

MODULE 9 · PHASE 2 · WEEKS 13–14

Personality

OpenStax ch. 11 · **IGNOU MPC-003: Personality: Theories and Assessment**

Why it matters: personality theories are also theories of how people change, and most counselling approaches grew directly out of one of them.

9.1 What "personality" means

Personality is a person's relatively **enduring** pattern of thinking, feeling and behaving, which makes them distinct and fairly consistent across situations. Every theory below answers: *what is personality made of, how does it develop, and how does it change?*

9.2 Psychodynamic approaches

Freud proposed that much of mental life is **unconscious**. The mind has three parts:

- **Id:** instinctual drives; pleasure principle.
- **Ego:** mediator; reality principle.
- **Superego:** internalised morals and ideals; source of guilt.

Psychosexual stages: oral → anal → phallic (Oedipus complex) → latency → genital. Fixation at a stage was thought to shape adult traits. (Much of this is not empirically supported, but the ideas of unconscious processes, early experience and defences remain influential.)

Defence mechanisms: still widely used in counselling language

Defence	What happens	Example
Repression	Pushing distressing material out of awareness	No memory of a painful period
Denial	Refusing to accept reality	"My drinking isn't a problem"
Projection	Attributing one's own unacceptable feelings to others	"My colleague is jealous of me" (when I'm jealous)
Displacement	Redirecting feelings to a safer target	Scolded by boss, snaps at spouse
Rationalisation	Plausible excuses for real motives	"I didn't want that promotion anyway"
Reaction formation	Acting the opposite of what one feels	Excessive niceness to someone resented
Regression	Returning to earlier behaviour under stress	Sulking, tantrums in an adult
Intellectualisation	Thinking abstractly to avoid feeling	Talking about a diagnosis only in statistics
Sublimation	Channelling impulses into acceptable outlets (considered mature)	Aggression into competitive sport

Neo-Freudians: Jung (collective unconscious, archetypes, introversion/extraversion; inspired the MBTI) · **Adler** (inferiority feelings and striving for superiority; birth order; social interest) · **Horney** (basic anxiety; moving toward, against or away from people; critiqued Freud's view of women) · **Erikson** (Module 8).

9.3 Humanistic approaches

- **Carl Rogers:** people have an **actualising tendency**. Distress arises from **incongruence** between the **self-concept** and actual experience, often created by **conditions of worth** ("I'm only lovable if

I top the class"). Healing comes from relationships offering **unconditional positive regard**, which is the basis of person-centred counselling (Module 12).

- **Abraham Maslow:** self-actualisation; peak experiences; studied healthy, fulfilled people rather than patients.

9.4 Social-cognitive approaches

- **Bandura: reciprocal determinism.** Person (thoughts, traits), behaviour and environment all influence each other.
- **Self-efficacy:** belief in one's ability to succeed at a specific task. Sources: (1) mastery experiences, the strongest; (2) vicarious experience (seeing others succeed); (3) verbal persuasion; (4) physiological and emotional states. Raising self-efficacy is central to helping people "stand on their own feet".
- **Rotter: locus of control.** *Internal* ("my actions shape outcomes") vs *external* ("luck, fate or others decide"). Internal is generally linked to better wellbeing, but cultural and structural realities matter. Not everything *is* in someone's control.
- **Mischel:** the "person–situation debate". Behaviour varies by situation in consistent "if…then…" patterns.

9.5 Trait approaches

Theorist	Model
Allport	Cardinal, central and secondary traits
Cattell	Factor analysis → 16 source traits (**16PF** questionnaire)
Eysenck	Three dimensions: Extraversion, Neuroticism, Psychoticism (PEN)
Big Five / Five-Factor Model (Costa & McCrae; Goldberg)	**O**penness · **C**onscientiousness · **E**xtraversion · **A**greeableness · **N**euroticism (OCEAN). The best-supported trait model. Moderately heritable, fairly stable in adulthood, found across many cultures. HEXACO adds Honesty-Humility.

> **About MBTI:** widely used in companies, but research finds weak reliability (people often get a different type on retest) and it forces continuous traits into boxes. As a workplace counsellor, prefer Big Five measures, and be ready to explain why politely.

9.6 Indian perspectives

- **Triguna theory** (from Sankhya philosophy and the Bhagavad Gita): personality reflects the balance of three *gunas*.
  - **Sattva:** clarity, calm, goodness, wisdom.
  - **Rajas:** activity, passion, ambition, restlessness.
  - **Tamas:** inertia, dullness, confusion.

  All three exist in everyone in varying proportions. Indian psychologists have built questionnaires such as the Vedic Personality Inventory (Wolf, 1998) and Indian adaptations.
- **Ayurvedic** *prakriti* (vata, pitta, kapha) is sometimes linked to temperament, though as a psychological model it lacks strong scientific validation.
- Concepts of **self in Indian thought**, such as the distinction between *ahamkara* (ego) and *atman* (deeper self), and the relational, family-embedded self, matter for culturally sensitive counselling.

9.7 Assessing personality

Type	Examples	Notes
Self-report inventories	**MMPI-2** (clinical; validity scales detect faking), **NEO-PI-R** (Big Five), 16PF, EPQ	Objective scoring; vulnerable to social desirability
Projective tests	**Rorschach** inkblots, **TAT** (Murray & Morgan; tell stories about ambiguous pictures), sentence completion	Rich but debated reliability and validity
Behavioural observation & interviews	Structured or unstructured interviews, ratings	Context-rich, time-consuming

Good tests are: reliable (consistent: test–retest, internal consistency, inter-rater), **valid** (measure what they claim: content, criterion, construct validity), and **standardised** with appropriate **norms**, ideally for Indian populations. In India, use of many clinical tests is restricted to RCI-licensed professionals. Counsellors should stay within their competence.

Try this

Take a free Big Five test (IPIP-based, link in sources). Write: which scores surprised you? How might your Neuroticism and Agreeableness scores affect you as a counsellor (e.g. over-absorbing clients' feelings, avoiding confrontation)?

Self-check

1. Describe the id, ego and superego, and name five defence mechanisms.
2. What are "conditions of worth" and "incongruence" in Rogers' theory?
3. What are the four sources of self-efficacy?
4. Name the Big Five. Why is the Big Five preferred over MBTI?
5. Explain the three gunas.
6. Distinguish reliability and validity.

Go deeper: OpenStax Psychology 2e, ch. 11 · Big Five · Defence mechanisms · Guna · Self-efficacy · Free Big Five test: openpsychometrics.org/tests/IPIP-BFFM · IGNOU MPC-003 assignment

MODULE 10 · PHASE 2 · WEEKS 15–16

Social Psychology

OpenStax ch. 12 · **IGNOU MPC-004: Advanced Social Psychology**

Why it matters: people suffer and heal in relationships, groups and cultures. Workplace problems are mostly social-psychological problems.

10.1 Social thinking: attribution

- **Attribution** = how we explain behaviour: *dispositional* (internal: personality) vs *situational* (external: circumstances). Heider founded the field.
- **Kelley's covariation model**: we use **consensus** (do others act the same?), **distinctiveness** (does this person act this way only here?) and **consistency** (always?).
- **Fundamental attribution error** (Ross): overestimating personality and underestimating the situation when judging *others* ("He's late because he's careless").
- **Actor–observer bias:** our *own* behaviour we blame on situations.
- **Self-serving bias:** success = me, failure = circumstances. (Depression often *reverses* this: failures = me.)
- FAE is weaker in collectivist cultures, including India, where situational and relational explanations come more naturally.

10.2 Attitudes and persuasion

- Attitudes have **ABC** components: Affective, Behavioural, Cognitive.
- **Cognitive dissonance** (Festinger; Festinger & Carlsmith, 1959): when actions and beliefs clash, we feel tension and often change the belief to fit the action. Paid $1 to lie that a dull task was fun, participants later rated it more enjoyable than those paid $20.
- **Elaboration Likelihood Model** (Petty & Cacioppo): **central route** (careful thought about arguments; durable change) vs **peripheral route** (cues like attractiveness, celebrity, social proof; fragile change).
- **Cialdini's principles of influence:** reciprocity, commitment and consistency, social proof, authority, liking, scarcity (later, unity).

10.3 Social influence

Study	Finding
Asch (1951): line-length conformity	About a third of responses conformed to an obviously wrong majority; ~75% conformed at least once. **Normative** influence (to fit in) vs **informational** influence (others might know better).
Milgram (1963): obedience	About 65% of participants went to the maximum "450-volt" shock when instructed by an authority figure. Obedience increases with proximity to authority and distance from the victim. Ethically controversial.
Zimbardo (1971): Stanford prison	Famous, but now heavily criticised for methodology and experimenter influence. Cite with caution.
Sherif: autokinetic effect; Robbers Cave	Group norms form quickly; intergroup conflict reduces with **superordinate goals**.

10.4 Groups

- **Social facilitation** (better on easy tasks when watched) vs **social loafing** (less effort when individual effort is hidden).

- **Deindividuation** in crowds and online anonymity.
- **Group polarisation:** discussion makes groups more extreme (echo chambers).
- **Groupthink** (Janis): cohesive groups suppress dissent and make poor decisions. The antidotes are psychological safety, a devil's advocate and outside input.
- **Bystander effect** (Darley & Latané, 1968): the more bystanders, the less likely anyone helps, through **diffusion of responsibility**. Relevant to workplace harassment and online crises.

10.5 Prejudice and intergroup relations

- **Stereotype** (belief) → **prejudice** (attitude) → **discrimination** (behaviour).
- **Social identity theory** (Tajfel & Turner): we derive self-esteem from groups and favour our **in-group**. Even arbitrary "minimal groups" trigger bias.
- **Implicit bias**: automatic associations we may not endorse.
- **Contact hypothesis** (Allport): contact reduces prejudice under equal status, common goals, cooperation and institutional support.
- **Indian context:** caste, religion, region, language, gender and sexuality shape experiences of discrimination and mental health. A counsellor must examine their own biases and recognise **minority stress**.

10.6 Relationships and prosocial behaviour

- **Attraction**: proximity, similarity, familiarity (the **mere-exposure effect**), reciprocity.
- **Sternberg's triangular theory of love**: intimacy, passion, commitment.
- **Altruism**: empathy–altruism hypothesis (Batson) vs social exchange; norms of reciprocity and social responsibility.
- **Aggression**: frustration–aggression hypothesis; social learning (Bandura).

10.7 Culture and the self

Individualism vs collectivism (Hofstede; Triandis): Markus & Kitayama (1991) distinguished the **independent self** (common in the West: unique, autonomous) from the **interdependent self** (common in Asia, including India: defined through relationships and roles). Indian psychologists such as J.B.P. Sinha described "**nurturant-task**" leadership in Indian organisations. For counselling, this means "becoming independent" may not be every client's goal. "Standing on your own feet" in India often means strength *within* the family and community, not apart from it.

> **Your UX edge:** Social proof, scarcity and defaults are persuasion patterns you've seen (or designed) in products. You can help clients notice the same pressures in their feeds and workplaces.

Try this

For one week, whenever someone annoys you, write your instant explanation (usually dispositional) and then two possible situational explanations. Notice how your feeling changes. This is the fundamental attribution error, and also basic cognitive reappraisal.

Self-check

1. Define the fundamental attribution error and the self-serving bias.
2. Explain cognitive dissonance using Festinger & Carlsmith's study.
3. Compare Asch's and Milgram's findings.
4. What is groupthink and how do you prevent it?
5. How does the interdependent self change how you'd counsel an Indian client?

Go deeper: OpenStax Psychology 2e, ch. 12 · Fundamental attribution error · Cognitive dissonance · Social identity theory · Bystander effect · IGNOU MPC-004 assignments and past papers: search "MPC-004" on ignou.ac.in

MODULE 11 · PHASE 2 · WEEKS 17–18

When the Mind Is in Distress

OpenStax ch. 15 · Year-2 counselling courses (MPCE-021/022)

*Why it matters: as a counsellor you won't diagnose severe illness, but you must **recognise** it, **know your limits**, and **refer** safely. Recognising risk can save a life.*

11.1 What counts as a "disorder"?

No single rule; clinicians weigh the "four Ds":

- **Deviance:** unusual for the person's culture.
- **Distress:** causes suffering.
- **Dysfunction:** interferes with work, relationships, self-care.
- **Danger:** to self or others.

Culture matters: hearing a deceased relative's voice may be a normal grief experience in some communities. **Classification systems**: the **DSM-5-TR** (American Psychiatric Association, 2022) and the **ICD-11** (WHO, in effect since 2022; used officially in India and much of the world). Explanatory frameworks: **biopsychosocial** and the **diathesis–stress model** (a vulnerability plus stress triggers a disorder).

11.2 Conditions you'll meet most often

Condition	Core features (simplified, not for diagnosis)
Depression (major depressive episode)	Low mood and/or loss of interest or pleasure (*anhedonia*) most of the day, nearly every day, for ≥2 weeks, plus symptoms like sleep or appetite change, fatigue, worthlessness or guilt, poor concentration, slowed or agitated movement, thoughts of death or suicide.
Generalised anxiety	Excessive, hard-to-control worry about many things, for months, with restlessness, tension, fatigue, irritability, poor sleep.
Panic disorder	Recurrent unexpected panic attacks (racing heart, breathlessness, fear of dying or losing control), plus ongoing worry about attacks or avoidance.
Social anxiety	Intense fear of being judged in social or performance situations; avoidance (e.g. of meetings, presentations).
Specific phobia	Intense fear of a specific object or situation
OCD	Obsessions (intrusive, unwanted thoughts) and compulsions (repetitive acts to reduce anxiety). Not the same as being "neat".
PTSD	After trauma: intrusive memories or nightmares, avoidance, negative changes in thoughts and mood, hyperarousal, lasting over a month. ICD-11 also has *Complex PTSD*.
Adjustment disorder	Distress or impairment out of proportion to an identifiable stressor (job loss, breakup), starting soon after it. Very common in counselling settings.
Bipolar disorder	Episodes of mania or hypomania (elevated or irritable mood, less need for sleep, racing thoughts, risky behaviour) and usually depression. **Needs psychiatric care.**
Psychotic disorders (e.g. schizophrenia)	*Positive* symptoms (hallucinations, delusions, disorganised speech) and *negative* symptoms (flat affect, withdrawal). **Needs psychiatric care.**
Substance use disorders	Loss of control, craving, continued use despite harm, tolerance and withdrawal. Alcohol is the biggest problem in India.
ADHD (adult)	Persistent inattention and/or hyperactivity-impulsivity since childhood; often first noticed at work.
Eating disorders	Anorexia, bulimia, binge-eating. Medical risk; needs a specialist team.
Personality disorders	Long-standing, inflexible patterns causing distress. Clusters A (odd), B (dramatic/emotional, e.g. borderline), C (anxious).
Somatic symptoms	Distress expressed through body complaints. Common in South Asia ("tension", headaches, fatigue).

11.3 Screening tools (free, widely used)

- **PHQ-9** (depression): 9 items scored 0–3; total 0–27. Cut-offs **5 / 10 / 15 / 20** = mild / moderate / moderately severe / severe. **Item 9 asks about thoughts of death or self-harm. Any positive answer needs follow-up.**
- **GAD-7** (anxiety): 7 items, total 0–21. Cut-offs **5 / 10 / 15** = mild / moderate / severe.
- Screening ≠ diagnosis. Tools guide a conversation and track progress; they don't label people.

11.4 Suicide: what every helper must know

Facts: India records well over a lakh suicides a year (NCRB reported about 1.7 lakh in 2022). Many people who die by suicide had contact with someone shortly before.

Myths vs facts

- ❌ "Asking about suicide puts the idea in their head." ✅ Research shows asking directly does **not** increase risk; it often brings relief.
- ❌ "People who talk about it won't do it." ✅ Most people give some warning. Take every mention seriously.
- ❌ "Only a professional can help." ✅ Anyone can listen, stay, and connect the person to help.

Warning signs: talking about wanting to die or being a burden; hopelessness; withdrawing; giving things away; saying goodbye; increased alcohol or drug use; extreme mood swings; sudden calm after deep depression; searching for means.

What to do (as a trainee or anyone): ask directly ("Are you thinking about killing yourself?") · listen without judgement · don't leave them alone if risk is immediate · remove access to means where possible · **connect to help: Tele-MANAS 14416, emergency services 112, or a hospital** · inform someone in their support network if risk is high · **never promise secrecy about risk**.

As a qualified counsellor you'll learn structured risk assessment and **safety planning** (Stanley & Brown). Never handle risk alone as a trainee. Involve your supervisor.

11.5 Know your scope: when to refer

Refer to…	When
Psychiatrist	Suicide risk, psychosis, mania, severe depression, need for medication, substance withdrawal
Clinical psychologist	Formal diagnosis, psychological testing, complex trauma, severe personality difficulties, eating disorders
Physician	Possible medical causes (thyroid, deficiencies, medication side effects)
Legal / social services	Domestic violence, harassment, child protection issues

Counselling psychologists typically work with adjustment, stress, mild-to-moderate anxiety and depression, relationship and career issues, and wellbeing, often *alongside* psychiatric care for more severe conditions.

11.6 Stigma and language

Use **person-first** language ("a person with schizophrenia", not "a schizophrenic") unless the person prefers otherwise. Avoid casual misuse ("I'm so OCD", "she's bipolar about it"). In India, stigma often stops people from seeking help, and how you talk is part of the treatment.

Try this

Fill in PHQ-9 and GAD-7 for yourself honestly (links below). This isn't for diagnosis, just to experience how screening feels from the client's chair. If scores are moderate or higher, or item 9 is positive, please talk to a professional. That's a strength, not a failure.

Self-check

1. What are the four Ds?
2. Which two classification systems exist, and which does India use officially?
3. Name the core features of depression and of panic disorder.
4. What are the PHQ-9 cut-offs? Which item needs special attention?
5. Give three myths and facts about suicide.
6. Name four situations that need referral to a psychiatrist.

Go deeper: OpenStax Psychology 2e, ch. 15 · PHQ-9 & GAD-7 (free): phqscreeners.com · ICD-11 · Diathesis–stress model · Tele-MANAS: telemanas.mohfw.gov.in · WHO PFA guide: who.int · Free PFA course: DisasterReady

MODULE 12 · PHASE 2 · WEEKS 19–21

Counselling Approaches

OpenStax ch. 16 · **IGNOU MPCE-021 (Counselling Psychology) & MPCE-023 (Interventions)**

Why it matters: this is the toolkit of your future profession. You don't need to master any approach yet, but you should know what each believes, what it does, and when it fits.

12.1 First, what all good counselling shares

Research on "common factors" suggests a large share of therapy's benefit comes from elements that every good approach has: a strong **therapeutic alliance** (bond + agreement on goals and tasks; Bordin), **empathy**, positive **expectations** (hope), a credible **rationale**, and the client's own resources. Different methods then add specific techniques. So your relationship skills (Module 13) matter as much as your choice of theory.

12.2 Person-centred counselling (Carl Rogers)

- **Belief:** people have an innate drive toward growth. Distress comes from incongruence between self-concept and experience, caused by conditions of worth (Module 9).
- **Core conditions** the counsellor offers: **empathy**, **unconditional positive regard**, **congruence** (genuineness). Rogers believed these were "necessary and sufficient" for change (today most see them as necessary but not always sufficient).
- **Style:** non-directive; the client leads; the counsellor reflects and clarifies. Client = expert on their own life.
- **Best for:** self-esteem, identity, feeling unheard. It's the foundation for all counselling training.

12.3 Psychodynamic and existential approaches

Psychodynamic counselling explores how unconscious patterns and early relationships show up now. Key concepts: **insight**, **defences**, **transference** (the client relating to the counsellor as to a past figure) and **countertransference** (the counsellor's own reactions, which are useful information if noticed). Short-term psychodynamic therapies exist with reasonable evidence.

Existential approaches: **Viktor Frankl's logotherapy** holds that the primary human drive is the will to *meaning*, found through work or creation, love and relationships, and the attitude we take to unavoidable suffering. **Irvin Yalom** describes four "ultimate concerns": **death, freedom (and responsibility), isolation, meaninglessness**. Very relevant for career changes, burnout ("what's the point?") and AI-era anxiety about purpose.

12.4 Cognitive Behavioural Therapy (CBT): in depth

CBT (Aaron Beck, 1960s) is the most researched talking therapy. Core idea: **our interpretations of events, not the events alone, shape our feelings and behaviour**, and those interpretations can be examined and changed.

Beck's levels of cognition

Level	What it is	Example
Core beliefs	Deep, global beliefs about self, others, world, often formed early	"I'm not good enough."
Intermediate beliefs	Rules, attitudes, assumptions ("if…then")	"If I'm not perfect, people will see I'm a fraud."
Automatic thoughts	Quick, situation-specific thoughts that pop up	"This design review will expose me."

Beck's **cognitive triad** in depression: negative views of the **self**, the **world**, and the **future**.

The CBT model of a moment

Situation → Automatic thought → Emotion + body sensations → Behaviour → (consequences reinforce the thought)

Thinking traps (cognitive distortions)

Trap	Sounds like	Balancing question
All-or-nothing	"If it's not perfect, it's a failure."	"Is there a middle ground?"
Overgeneralisation	"I *always* mess up."	"Always? What are the exceptions?"
Mental filter	Fixating on one criticism	"What am I leaving out?"
Discounting the positive	"That praise doesn't count."	"Would I discount it for a friend?"
Mind reading	"She thinks I'm useless."	"What's the evidence? Could I ask?"
Fortune telling	"This will go badly."	"What else could happen?"
Catastrophising	"My life will be ruined."	"Worst, best, most likely outcome?"
Emotional reasoning	"I feel useless, so I am."	"Feelings aren't facts."
Should statements	"I should be further ahead."	"Says who? What would I prefer?"
Labelling	"I'm a loser."	"What did I do, not what am I?"
Personalisation	"It's all my fault."	"What other factors contributed?"

Core CBT techniques

- **Thought record:** situation → emotion (0–100%) → automatic thought → evidence for → evidence against → balanced thought → re-rate emotion.
- **Socratic questioning:** guided discovery through questions, not telling ("What would you say to a friend who thought that?").
- **Behavioural experiments:** testing a belief in real life ("If I speak up in the meeting, people will laugh." Try it and see).
- **Behavioural activation** (especially for depression): schedule small activities that bring *pleasure* or *mastery*. Action comes before motivation, not after.
- **Exposure** (for anxiety): face feared situations gradually using a **fear hierarchy** rated in SUDS (Subjective Units of Distress, 0–100), staying until anxiety drops, without "safety behaviours". This reverses avoidance learning (Module 4).
- **Problem-solving** training, **relaxation**, **homework**, and session **structure** (agenda, review, homework).

Variants: **CBT-I** (insomnia), trauma-focused CBT, and computerised or app-based CBT, which is another bridge to your design background.

12.5 Rational Emotive Behaviour Therapy (REBT): Albert Ellis

Predates Beck's CBT. The **ABCDE model**:

	Step	Example
A	**Activating event**	Not shortlisted for a job
B	**Belief** (irrational)	"I *must* succeed; it's awful and I'm worthless if I don't."
C	**Consequence** (emotional/behavioural)	Depression, stops applying
D	**Disputing**	"Where's the law that says I must? Does one rejection make me worthless?"
E	**Effective new belief**	"I'd strongly prefer success. This is disappointing, not catastrophic. I can keep trying."

Ellis targets "musts" (demandingness), "awfulising", "I-can't-stand-it-itis" and global self-rating. He advocated **unconditional self-acceptance**.

12.6 Brief and "third-wave" approaches

Solution-Focused Brief Therapy (SFBT): de Shazer & Insoo Kim Berg

Focuses on solutions and strengths, not problem history. Few sessions. Techniques:

- **Miracle question:** "Suppose tonight, while you're asleep, a miracle happens and the problem is solved. When you wake up, what's the first small thing you'd notice that tells you things are different?"
- **Exception questions:** "When is the problem less bad? What's different then?"
- **Scaling questions:** "On 0–10, where are you now? What would one point higher look like?"
- **Coping questions:** "How have you managed to keep going despite this?"

Very suitable for workplace and EAP counselling (short, practical).

Acceptance and Commitment Therapy (ACT): Steven Hayes

Goal: **psychological flexibility**. Instead of fighting thoughts, change your *relationship* to them and act on values. Six core processes (the "hexaflex"):

1. **Acceptance:** making room for difficult feelings instead of struggling.
2. **Cognitive defusion:** seeing thoughts as thoughts ("I'm having the thought that I'm a failure").
3. **Present-moment awareness.**
4. **Self-as-context:** the observing self that notices experiences.
5. **Values:** what truly matters to you.
6. **Committed action:** steps guided by values.

Great for chronic stress, burnout, and "Sunday dread" at work.

Motivational Interviewing (MI): Miller & Rollnick

A collaborative style for strengthening a person's own motivation to change, originally for addiction. Spirit: **partnership, acceptance, compassion, evocation**. Core skills: **OARS**: **O**pen questions, **A**ffirmations, **R**eflections, **S**ummaries. The counsellor listens for and strengthens **change talk** and rolls with "sustain talk" instead of arguing.

Pairs with the **Transtheoretical Model (Stages of Change)**, from Prochaska & DiClemente: **precontemplation → contemplation → preparation → action → maintenance** (relapse is normal and part of the cycle). Match your approach to the stage. Pushing action on someone in contemplation backfires.

Mindfulness-based approaches

MBSR (Jon Kabat-Zinn, 8-week stress reduction) and **MBCT** (mindfulness-based cognitive therapy, evidence for preventing depressive relapse). Mindfulness = paying attention, on purpose, in the present moment, without judgement. It has roots in Buddhist and Indian contemplative traditions, which can make it culturally familiar to Indian clients.

Others to recognise

- **Narrative therapy** (White & Epston): "the person is not the problem; the problem is the problem". **Externalising** ("How does the Anxiety convince you to stay quiet?"), re-authoring life stories.
- **Gestalt therapy** (Perls): here-and-now awareness, the **empty chair** technique.
- **Family systems therapy**: problems are seen within family patterns. Very relevant in India.
- **DBT** (Linehan): skills in mindfulness, distress tolerance, emotion regulation and interpersonal effectiveness; developed for borderline personality disorder.
- **Positive psychology interventions**: gratitude, strengths (VIA), savouring.
- **Indian and integrative approaches**: yoga-based interventions (evidence growing, e.g. at NIMHANS), and the guru–chela relationship as a culturally congruent model (J.S. Neki).

12.7 Comparison at a glance

Approach	Focus	Counsellor's role	Typical length	Fits well with…
Person-centred	Self-acceptance, growth	Empathic companion	Open	Self-esteem, being unheard
Psychodynamic	Unconscious patterns, past	Interpreter, reflective	Medium–long	Repeating relationship patterns
Existential	Meaning, choice	Fellow traveller	Open	Purpose crises, career change
CBT	Thoughts & behaviours now	Collaborative coach	6–20 sessions	Anxiety, depression, insomnia
REBT	Irrational beliefs	Active disputer	Short–medium	Perfectionism, anger
SFBT	Solutions, exceptions	Curious questioner	1–6 sessions	EAP, practical goals
ACT	Flexibility, values	Guide	Short–medium	Chronic stress, burnout
MI	Ambivalence about change	Partner, evoker	1–4+ sessions	Habits, substance use, tech overuse

Most experienced counsellors become **integrative**: a stable base (often person-centred) plus techniques chosen for the client.

Try this

(1) Answer the **miracle question** for your own career change in writing. (2) Do one **ABCDE** on a recent upset. (3) Pick one value (e.g. "being useful to people") and write one committed action for this week (ACT).

Self-check

1. What are the "common factors" in therapy?
2. Name Rogers' three core conditions.
3. Explain Beck's three levels of cognition and the cognitive triad.
4. Walk through ABCDE with your own example.
5. State the miracle question and one scaling question.
6. Name ACT's six processes.
7. What does OARS stand for? List the stages of change.

Go deeper: OpenStax Psychology 2e, ch. 16 · Burns, *Feeling Good* · Yalom, *The Gift of Therapy* · Frankl, *Man's Search for Meaning* · Beck Institute · REBT · SFBT · ACT · Motivational interviewing · Stages of change · IGNOU MPCE-021 assignment (2026–27)

MODULE 13 · PHASE 2 · WEEKS 22–23

The Counselling Process & Micro-skills

IGNOU MPCE-021/023/024 (practicum)

Why it matters: theory tells you why; skills are what you actually do for 50 minutes with a person in front of you.

13.1 The arc of counselling

Stage	What happens
1. Referral & first contact	Fit, fees, availability, emergency check
2. Intake / assessment	Presenting problem, history, current functioning, supports, **risk**, expectations, screening tools
3. Contracting & informed consent	Confidentiality and its limits, session length (often ~50 min), frequency, fees, cancellations, records, online norms
4. Building the relationship	Rapport, trust, safety
5. Case conceptualisation & goals	A shared understanding; SMART goals
6. Intervention / working phase	Applying approaches; homework; reviewing progress
7. Evaluation	Re-scoring measures; client feedback
8. Termination & follow-up	Planned ending, consolidating gains, relapse prevention

13.2 Egan's Skilled Helper model

A widely taught three-stage problem-management model:

1. **Current picture**: "What's going on?" Help the client tell their story, find blind spots, and choose what to work on (leverage).
2. **Preferred picture**: "What do I want instead?" Possibilities, then realistic goals and commitment.
3. **The way forward**: "How do I get there?" Strategies, the best-fit plan, action.

It maps neatly onto design thinking: discover → define → develop → deliver. You'll find this model natural.

13.3 Micro-skills (Ivey's hierarchy, simplified)

Skill	Purpose	Example
Attending behaviour	Show you're with them: eye contact (culturally appropriate), open posture, vocal tone, following their topic. Egan's **SOLER**: Sit squarely, Open posture, Lean in, Eye contact, Relaxed.	Nodding, "mm-hmm", phone away
Open questions	Invite elaboration	"What was that like for you?" / "How did you respond?"
Closed questions	Specific facts (use sparingly)	"How long have you been at this company?"
Encouragers	Keep the flow	"Go on…" or repeating a key word: "Exhausted?"
Paraphrasing	Reflect content in fresh words	"So the deadlines keep moving, and you're the one absorbing it."
Reflection of feeling	Name the emotion underneath	"It sounds like you feel invisible, and maybe a bit angry."
Reflection of meaning	Connect to values	"Being trusted really matters to you."
Summarising	Pull threads together	"Let me check I've understood the main things…"
Silence	Room to think and feel	Waiting 5–10 seconds
Immediacy	Talk about what's happening between you now	"I notice you changed the topic when we got to your father."
Challenging / confrontation	Gently point out discrepancies	"You say it doesn't matter, and your eyes filled up as you said it."
Self-disclosure (rare, purposeful)	Only if it serves the client	"Many people I've worked with find…"

Common beginner mistakes: asking "why" questions (they feel accusatory; try "what led to…"), stacking several questions, giving advice too early, rescuing, interrupting silence, making it about yourself, false reassurance ("Don't worry, it'll be fine").

13.4 Case conceptualisation: the 5 Ps

P	Question	Example (burnt-out product manager)
Presenting	What's the problem now?	Exhaustion, irritability, insomnia, dread on Sundays
Predisposing	What made them vulnerable?	High achiever; love felt conditional on grades growing up
Precipitating	What triggered it?	Reorg doubled their team; new manager
Perpetuating	What keeps it going?	Can't say no; works nights; stopped exercising; belief "rest = laziness"
Protective	What helps?	Supportive partner, enjoys music, insight, financial cushion

The 5 Ps turn a story into a plan: interventions target **perpetuating** factors and build on **protective** ones.

13.5 Goals and progress

Goals should be **SMART** (specific, measurable, achievable, relevant, time-bound) and *owned by the client*. Track with scales (PHQ-9, GAD-7, 0–10 ratings) and by asking clients for feedback each session. Routine outcome monitoring improves results.

13.6 The counsellor as a person

- **Self-awareness:** know your triggers, biases and "hot topics" (e.g. career pressure, family expectations).
- **Countertransference:** e.g. feeling irritated with a client who reminds you of someone. Notice it, take it to supervision.
- **Supervision:** a qualified supervisor reviews your work. It's essential throughout a counselling career, not just in training.
- **Boundaries:** time, contact outside sessions, gifts, social media, dual relationships (Module 14).
- **Cultural humility:** curiosity about each client's world (language, caste, religion, gender, sexuality, class), rather than assuming.
- **Online counselling:** check identity and location, have an emergency plan, ensure privacy on both ends, use secure platforms.

> **Your UX edge, and a trap:** User interviews built your listening, open questions and neutrality. But in UX you *extract insight for a product* and then solve the problem yourself. In counselling, the insight belongs to the client, and the solution must be theirs. Resist the designer's urge to "fix it". Your job is to help them see clearly and choose.

Try this
In three ordinary conversations this week, use only open questions, paraphrases and reflections of feeling for five minutes (no advice, no stories of your own). Afterwards, note what the other person said that you'd normally have missed.

Self-check

1. List the stages of the counselling process.
2. What are Egan's three stages?
3. Give an example each of a paraphrase, a reflection of feeling, and a summary.
4. Why avoid "why" questions?
5. Explain the 5 Ps with your own example.
6. What is countertransference and what do you do about it?

Go deeper: Gerard Egan, *The Skilled Helper* · Allen Ivey et al., *Intentional Interviewing and Counseling* · Microcounseling · Therapeutic relationship · Esther Perel's *Where Should We Begin?* (listen for the skills)

MODULE 14 · PHASE 2 · WEEK 24

Ethics & the Law in India

IGNOU MPCE-021 · Professional practice

Why it matters: clients trust you with their most private selves. Ethics protect them, and protect you.

14.1 Core ethical principles

Principle	Meaning
Autonomy	Respect the client's right to make their own choices
Beneficence	Act for the client's good
Non-maleficence	Do no harm
Justice	Fairness; no discrimination; access
Fidelity	Keep promises; be trustworthy
Veracity	Be honest

(Based on Kitchener's model, used across counselling ethics codes such as those of the ACA and BACP. Indian professional bodies publish their own codes.)

14.2 Confidentiality and its limits

What clients say stays private, **except** when:

- There is **serious risk of harm to self** (suicide) or **to others**.
- A **child** is at risk of abuse. Under **POCSO, 2012**, anyone who learns of a sexual offence against a child must report it to the police or Special Juvenile Police Unit. Failure to report is itself an offence.
- A **court** orders disclosure.
- The client gives **written consent** (e.g. to share with their psychiatrist).
- Within **supervision** (anonymised as far as possible).

Explain these limits **in the first session**, as part of informed consent. In workplace (EAP) counselling, be especially clear: **the employer does not get individual information**, only anonymised usage data.

14.3 Other key ethical areas

- **Informed consent:** nature of counselling, risks and benefits, fees, alternatives, recording, right to stop.
- **Competence:** work only within your training. Refer when out of depth (Module 11).
- **Dual relationships:** avoid being a client's friend, colleague, business partner or romantic partner. Sexual relationships with clients are always unethical. In close-knit communities, manage unavoidable overlaps carefully.
- **Records:** accurate, secure, minimal; know how long to keep them.
- **Advertising:** honest titles. **Don't call yourself a "psychologist" or "therapist" before you're qualified and registered.**
- **Self-care and impairment:** don't practise when your own state could harm clients.
- **Technology:** secure video platforms, no client chats on personal WhatsApp if avoidable, encrypted notes.

14.4 Indian laws and regulations to know

Law / body	What it means for you
Mental Healthcare Act, 2017	Rights-based law. Every person has a **right to access** mental healthcare and to **confidentiality**; allows **advance directives** and **nominated representatives**; **Section 115** presumes a person who attempts suicide is under severe stress and shouldn't be punished (effectively decriminalising attempted suicide). Note: the Act's definition of "mental health professional" lists psychiatrists, clinical psychologists, psychiatric nurses and psychiatric social workers. Counsellors aren't named, so stay within counselling scope.
NCAHP Act, 2021 and 2026 registration rules	Counselling psychologists come under the National Commission for Allied and Healthcare Professions. Registration valid 5 years; 15 hrs/yr CPD.
Rehabilitation Council of India (RCI) Act, 1992	Licenses clinical psychologists (and some rehabilitation professionals). Using RCI-restricted titles or tests without registration isn't allowed.
POCSO Act, 2012	Mandatory reporting of child sexual abuse (relevant if you work with students or adolescents).
POSH Act, 2013	Prevention of sexual harassment at work. Internal Committees. Relevant to workplace counselling and referrals.
Digital Personal Data Protection Act, 2023	Client data is personal data. Collect with consent, store securely, use only for stated purposes.
Telemedicine Practice Guidelines, 2020	For doctors, but they shaped norms for tele-health. Counsellors should follow similar good practice.
Rights of Persons with Disabilities Act, 2016	Recognises mental illness as a disability; non-discrimination at work.
Transgender Persons (Protection of Rights) Act, 2019; Navtej Singh Johar judgment, 2018	Affirmative, non-discriminatory practice with LGBTQ+ clients. "Conversion therapy" is condemned by professional bodies and has been held to be professional misconduct (e.g. by the National Medical Commission for doctors).

14.5 An ethical decision-making model

1. Identify the problem (ethical, legal, clinical?).
2. Check codes and laws.
3. Consult your supervisor or a colleague.
4. List options and consequences for everyone involved.
5. Choose and act.
6. Document your reasoning.
7. Reflect afterwards.

Try this: dilemmas
What would you do? (a) A friend asks you to "just talk to" their cousin who seems depressed. (b) An EAP client's HR manager calls asking whether the client is "okay to handle a promotion". (c) A client sends you a friend request on Instagram. Write your answer, then compare with the principles above. (Suggested responses in Part F.)

Self-check

1. Name five ethical principles.
2. List the main exceptions to confidentiality.
3. What does Section 115 of the MHCA 2017 say?
4. Which law requires reporting child sexual abuse?
5. Why must EAP counsellors be extra clear about confidentiality?

Go deeper: Mental Healthcare Act, 2017 · POCSO Act · POSH Act · DPDP Act 2023 · NCAHP: ncahp.abdm.gov.in · Psychologs on NCAHP

MODULE 15 · PHASE 3 · WEEKS 26–27

Research Methods & Statistics

OpenStax ch. 2 · **IGNOU MPC-005 (Research Methods) & MPC-006 (Statistics)**

Why it matters: two of six first-year courses are research and statistics. Many arts-background students struggle here, and as an engineer you can turn it into your strongest subject.

15.1 The research toolkit

Method	What it does	Strength	Limitation
Case study	In-depth study of one person or group	Rich detail, rare cases	Can't generalise
Naturalistic observation	Watching behaviour in real settings	Realistic	No control; observer effects
Survey	Questionnaires to many people	Large samples, cheap	Self-report bias, wording effects
Correlational	Measures how two variables relate	Studies things you can't manipulate	**Correlation ≠ causation** (third variables, direction problem)
Experiment	Manipulates an IV, measures a DV, with random assignment	Can show **cause and effect**	May be artificial
Quasi-experiment	Like an experiment but no random assignment (e.g. comparing existing groups)	Practical	Weaker causal claims
Longitudinal / cross-sectional	Same people over time / different ages at one time	Development over time / quick	Costly, dropout / cohort effects
Qualitative	Interviews, focus groups, thematic analysis	Meaning, experience, "why"	Smaller samples, interpretation

15.2 Key terms

- **Hypothesis:** a testable prediction. **Null hypothesis (H_0):** no effect or difference. **Alternative (H_1):** there is one.
- **Independent variable (IV):** what you manipulate. **Dependent variable (DV):** what you measure.
- **Operational definition:** how exactly a concept is measured (e.g. "anxiety" = GAD-7 score).
- **Confounding variable:** an uncontrolled factor that offers an alternative explanation.
- **Control group, random assignment, placebo, double-blind.**
- **Population vs sample;** sampling methods: **random, stratified, systematic, cluster** (probability) vs **convenience, purposive, snowball, quota** (non-probability).
- **Internal validity** (did the IV really cause the DV?) vs **external validity** (does it generalise?).
- **Research ethics:** informed consent, right to withdraw, confidentiality, minimising harm, debriefing (especially after deception), ethics committee approval.

15.3 Qualitative research: you've already done it

Thematic analysis (Braun & Clarke, 2006) has six phases: (1) familiarise with data, (2) generate initial codes, (3) search for themes, (4) review themes, (5) define and name themes, (6) write up. If

you've done affinity mapping of interview notes, you've done a version of this. Other approaches: grounded theory, interpretative phenomenological analysis (IPA), narrative analysis, discourse analysis.

15.4 Statistics: the essentials

Scales of measurement (NOIR)

Scale	Property	Example
Nominal	Categories only	Gender, department
Ordinal	Order, unequal gaps	Rank, Likert item (strictly)
Interval	Equal gaps, no true zero	Temperature °C, IQ
Ratio	Equal gaps + true zero	Reaction time, income

Descriptive statistics

- **Central tendency:** mean (average; sensitive to outliers), median (middle value), mode (most frequent).
- **Variability:** range, variance, **standard deviation (SD)** (typical distance from the mean).
- **Normal distribution:** bell-shaped. About **68%** of scores fall within ±1 SD, **95%** within ±2 SD, **99.7%** within ±3 SD.
- **z-score** = (score − mean) / SD: how many SDs a score is from the mean. IQ 130 → z = (130−100)/15 = +2.
- **Skewness** (lopsided distribution) and **kurtosis** (peakedness).
- **Percentiles**: the percentage of scores below a given score.

Relationships

- **Correlation coefficient (r)**: −1 to +1. Sign = direction, size = strength. Pearson's r (interval/ratio), Spearman's rho (ranks).
- **Regression**: predicting one variable from another.

Inferential statistics: is the effect real?

- **p-value**: the probability of getting results at least this extreme if H_0 were true. Conventionally, $p < .05$ = "statistically significant".
- **Type I error** (false positive: rejecting a true H_0; probability = α) vs **Type II error** (false negative: missing a real effect; probability = β). **Power** = 1 − β.
- **Effect size**: how big the effect is. Cohen's d ≈ 0.2 small, 0.5 medium, 0.8 large. "Significant" isn't the same as "important".
- **Parametric tests** (assume roughly normal data, interval/ratio):
  - **t-test**: compares 2 means (independent or paired).
  - **ANOVA**: compares 3+ means (F-ratio); post-hoc tests find where differences lie.
  - Correlation / regression.
- **Non-parametric tests** (ordinal data or non-normal): **chi-square** (frequencies in categories), **Mann–Whitney U** (2 independent groups), **Wilcoxon** (paired), **Kruskal–Wallis** (3+ groups).

Your question	Likely test
Do women and men differ in burnout scores?	Independent t-test (or Mann–Whitney)
Did anxiety drop from before to after a workshop?	Paired t-test (or Wilcoxon)
Do three departments differ in stress?	One-way ANOVA (or Kruskal–Wallis)
Is screen time related to sleep quality?	Pearson / Spearman correlation
Is help-seeking (yes/no) related to gender?	Chi-square

15.5 Reading research critically

Ask: sample size and who was studied · control group? · random assignment? · effect size, not just p · replicated? · pre-registered? · conflicts of interest? · systematic reviews and **meta-analyses** sit near the top of the evidence pyramid; anecdotes and expert opinion at the bottom.

> **Your UX edge:** A/B tests are experiments; usability metrics are DVs; you know about sample sizes and statistical significance. And your engineering maths is more than enough for MPC-006.

Try this

Design a tiny study: "Does a 10-minute breathing exercise before work reduce afternoon stress?" Write the hypothesis, IV, DV (and how you'd measure it), design, sample, possible confounds, ethics, and which test you'd use.

Self-check

1. Why can't correlational studies show causation?
2. Define IV, DV, operational definition and confounding variable.
3. What percentage of scores fall within ±1 SD in a normal distribution?
4. Calculate the z-score for IQ 85.
5. Distinguish Type I and Type II errors.
6. Which test compares 3+ group means?
7. List the six phases of thematic analysis.

Go deeper: OpenStax Psychology 2e, ch. 2 · Khan Academy: Statistics & Probability · OpenStax Introductory Statistics · Thematic analysis · IGNOU assignments: MPC-005 (2026–27) · MPC-006 (2026–27)

MODULE 16 · PHASE 3 · WEEKS 28–29 · YOUR NICHE

Workplace Counselling & Wellbeing

OpenStax ch. 13 (Industrial-Organisational) · Year-2 specialisation and career

Why it matters: this is one of your chosen specialisations, and your corporate experience is a real advantage. You know the meetings, deadlines, reorgs and Slack pings first-hand.

16.1 Burnout

WHO (ICD-11): burnout is a syndrome resulting from **chronic workplace stress that has not been successfully managed**, with three dimensions:

1. **Exhaustion**: energy depletion.
2. **Mental distance / cynicism**: negativism about the job.
3. **Reduced professional efficacy.**

It's classed as an **occupational phenomenon, not a medical condition**, and applies only to work. These dimensions come from **Christina Maslach's** research and the **Maslach Burnout Inventory (MBI)**. Maslach and Leiter identified six workplace drivers: **workload, control, reward, community, fairness, values**. Burnout overlaps with depression, so always screen for both.

16.2 Models of work stress

Model	Core idea	Counselling implication
Job Demands–Resources (JD-R) (Demerouti, Bakker et al.)	Demands (workload, emotional labour, always-on messaging) drain energy → burnout. Resources (autonomy, support, feedback, growth) build motivation → engagement. Resources buffer demands.	Help clients reduce demands *and* grow resources; advise organisations similarly
Demand–Control (Karasek)	The highest strain is high demands with low control ("high-strain jobs"). Social support added later.	Increase sense of control and choice where possible
Effort–Reward Imbalance (Siegrist)	High effort with low reward (pay, esteem, security) → stress, especially with "over-commitment"	Explore over-commitment patterns and fairness
Psychological safety (Amy Edmondson)	A shared belief that it's safe to take interpersonal risks (speak up, admit mistakes). Google's "Project Aristotle" found it the top factor in team effectiveness.	Coach managers; help clients find voice

16.3 The WHO guidelines on mental health at work (2022)

WHO and ILO recommend organisational interventions (reducing psychosocial risks like excessive workload), **training managers** to support mental health (a first-ever WHO recommendation), worker training in mental health literacy, individual interventions (e.g. stress management), and support for return to work and for people with mental health conditions. WHO and ILO estimate that depression and anxiety cost about **12 billion working days** a year globally.

16.4 Employee Assistance Programmes (EAPs)

- Employer-paid, **confidential**, usually short-term counselling (often a few sessions per issue), delivered by external providers, increasingly via apps and phone or video. Many large Indian companies offer one.
- Typical issues: work stress, conflict with managers, performance anxiety, relationship or family problems, grief, financial stress, substance use, and crisis after incidents (e.g. layoffs, a colleague's

death).

- Skills needed: brief models (SFBT, CBT, MI), fast assessment, risk screening, referral networks, and very clear confidentiality boundaries.
- Other workplace roles: wellness workshops, manager training, critical-incident support, return-to-work support, designing wellbeing programmes (where your UX skills shine).

16.5 Common workplace issues

Issue	Notes
Impostor phenomenon (Clance & Imes, 1978)	Feeling like a fraud despite evidence of competence. Common in high achievers and in people who are "firsts" in their families. Not a diagnosis.
Perfectionism	Adaptive (high standards) vs maladaptive (fear of mistakes, self-criticism). REBT and CBT help.
Presenteeism	Working while unwell. Often costs more than absenteeism.
Job insecurity, layoffs, **AI anxiety**	Uncertainty is a major stressor. Counselling can address appraisal, identity beyond the job, practical planning and meaning.
Difficult managers, toxic cultures, bullying	Validate; avoid only "fix yourself" framing; options include boundaries, documentation, HR, exit.
Sexual harassment	Know the **POSH Act, 2013** process (Internal Committee); support and refer.
Work–life boundaries; remote work; "always-on"	Boundary theory: segmenters vs integrators. Right-to-disconnect debates.
Long hours culture	Common in Indian IT and startups. Consider cultural and economic realities, not just individual habits.
Career dilemmas	See 16.6.

16.6 Career counselling theories

- **Holland's RIASEC:** six interest types: **R**ealistic, **I**nvestigative, **A**rtistic, **S**ocial, **E**nterprising, **C**onventional. Satisfaction is higher when person and environment match.
- **Super's life-span, life-space theory:** career stages: growth, exploration, establishment, maintenance, disengagement. Career is one of several **life roles** (worker, partner, child, citizen...). Self-concept drives career choices.
- **Savickas's career construction theory:** careers as stories; **career adaptability** (concern, control, curiosity, confidence).
- **Schein's career anchors:** e.g. technical competence, autonomy, security, service/dedication to a cause, challenge, lifestyle.
- **Krumboltz's planned happenstance:** make use of unplanned events through curiosity, persistence, flexibility, optimism and risk-taking.
- Indian considerations: family's role in career decisions, prestige hierarchies (engineering, medicine), financial obligations, gender expectations.

> **Your UX edge:** Wellbeing programmes are products. They need user research, journey mapping, adoption design and measurement. Few counsellors can design them; few designers understand mental health. You could do both, which is a strong niche for consulting.

Try this

(1) Rate your own job on JD-R: list your top 5 demands and top 5 resources. (2) Identify your Holland code (top 3 letters) and your career anchor. (3) Notice how these explain your own move toward counselling (hint: Social + Investigative, "service" anchor?).

Self-check

1. State the WHO definition of burnout and its three dimensions.
2. Explain the JD-R model in your own words.
3. What did Karasek and Siegrist each propose?
4. What is psychological safety?
5. What is an EAP, and what are its confidentiality rules?
6. Name Holland's six types and Super's career stages.

Go deeper: OpenStax Psychology 2e, ch. 13 · WHO on burnout · WHO/ILO: mental health at work (2022) · WHO guidelines on mental health at work (full text, NCBI) · JD-R model · Psychological safety · Holland Codes · Impostor phenomenon · Podcasts: WorkLife, How's Work? · Book: Nagoski, *Burnout*

MODULE 17 · PHASE 3 · WEEKS 30–31 · YOUR NICHE

Digital-Age Wellbeing

An emerging field: builds on Modules 3, 4, 7, 10, 12

Why it matters: this is the reason you gave for this career, that the AI and digital age will increase people's need for human help. Here you are an insider.

17.1 How products capture attention

- **The attention economy:** when a service is free, user attention and data are what's sold, so engagement metrics drive design.
- **Fogg Behaviour Model: B = MAP.** Behaviour happens when **M**otivation, **A**bility and a **P**rompt converge. Products make actions effortless (Ability) and send well-timed prompts.
- **Hooked model** (Nir Eyal): **trigger → action → variable reward → investment**. Triggers shift from external (notifications) to internal (boredom, loneliness, anxiety).
- **Variable rewards** (Module 4): feeds, likes, matches. Unpredictable rewards are the most habit-forming.
- **Dark patterns / deceptive design:** infinite scroll, autoplay, streaks that punish breaks, "confirmshaming", hard-to-cancel subscriptions. India's Central Consumer Protection Authority issued guidelines against dark patterns in 2023.
- **Lembke's pleasure–pain balance** (*Dopamine Nation*): repeated easy pleasure is followed by a compensatory "pain" side (craving, low mood). Over time the baseline shifts. Strategies: self-binding (barriers), abstinence periods (she often suggests ~30 days), radical honesty, and seeking healthy "pain" (exercise, cold water, effortful tasks).

17.2 What the evidence says (be nuanced)

- Large studies often find **small average associations** between overall screen time and wellbeing (e.g. Orben & Przybylski, 2019). *What* people do, *why*, and *what it displaces* (sleep, exercise, in-person time) matter more than total hours.
- Jonathan Haidt (*The Anxious Generation*) argues smartphones and social media drove a sharp rise in adolescent anxiety and depression after ~2012. Others (e.g. Candice Odgers) argue the causal evidence is weaker. **A good counsellor holds both:** take the individual client's experience seriously while avoiding moral panic.
- Clearer harms: **sleep disruption** (late-night use), **social comparison** (Festinger's theory; curated feeds invite upward comparison), **cyberbullying**, **problematic use** displacing important activities.

17.3 Clinical concepts

Concept	Status	Features
Gaming disorder	Official diagnosis in **ICD-11**	Impaired control over gaming, gaming taking priority over other interests and activities, continuing despite harm, significant impairment, usually evident over ~12 months
Problematic smartphone / internet / social media use	Not a formal diagnosis; widely researched	Similar pattern: loss of control, conflict, withdrawal-like irritability, harm
FOMO (fear of missing out)	Psychological construct (Przybylski et al., 2013)	Worry that others are having rewarding experiences without you; linked to compulsive checking
Technostress	Organisational construct	Techno-overload, techno-invasion (work into home), techno-complexity, techno-insecurity (fear of being replaced), techno-uncertainty
Bedtime procrastination	Research construct	Delaying sleep without good reason, often on phones ("revenge bedtime procrastination")
Cyberchondria	Research construct	Health anxiety fuelled by online symptom-searching

17.4 Loneliness, AI and the future of work

- **Loneliness**: the US Surgeon General's 2023 advisory (*Our Epidemic of Loneliness and Isolation*) linked social disconnection to health risks comparable to well-known risk factors, and WHO launched a Commission on Social Connection (2023). Digital connection can help or replace real connection; quality matters.
- **AI companions and chatbots**: can offer comfort and practice, but raise concerns about emotional dependency, reduced human contact, privacy, and unsafe responses in crises (reports of harm have emerged). Mental-health chatbots (e.g. CBT-based apps) show some evidence for mild symptoms but are not a replacement for human care in serious cases.
- **AI and work anxiety**: fear of job loss, deskilling, identity threat ("If AI can do my job, who am I?"). Counselling can address appraisal (realistic vs catastrophic), identity beyond the role, adaptability and skill-building, and meaning (existential lens, Module 12).
- **Information overload** and doom-scrolling during crises increase anxiety.

17.5 Counselling for digital wellbeing: a toolkit

1. **Assess without judgement:** what does the person use, when, how they feel before and after, what need it meets (boredom, loneliness, anxiety, escape).
2. **Motivational interviewing** for ambivalence ("Part of you enjoys it, part of you feels drained").
3. **Functional analysis** (cue → behaviour → reward → cost), as in Module 4.
4. **Environmental redesign** (add friction): grayscale mode, delete apps from the home screen, notifications off, phone outside the bedroom, app timers, website blockers. Implementation intentions: "If I reach for my phone in bed, then I'll read two pages instead."
5. **Replace, don't just remove:** plan alternative rewards that meet the same need (a call with a friend, a walk, a hobby).
6. **Values-based use** (ACT): "Which uses serve what matters to me?" (Newport's *Digital Minimalism*: a 30-day "digital declutter", then reintroduce only what adds value).
7. **Sleep hygiene** and CBT-I principles.

8. **Social reconnection**: scheduling real-world contact; addressing social anxiety where present.
9. **Workplace level:** communication norms (response-time expectations, no-meeting blocks, after-hours policies).
10. For gaming disorder or severe problems, refer to specialist services. **NIMHANS's SHUT Clinic** (Service for Healthy Use of Technology) is a pioneering Indian example.

> **Your UX edge, your signature niche:** "Design-informed counselling". You can explain to clients *exactly* how a product is built to keep them, which replaces shame with strategy. You can also consult companies on humane design and digital-wellbeing features, and contribute to mental-health apps. The Center for Humane Technology was founded by former tech designers for the same reason.

Try this: 7-day digital audit

Each day, check your screen-time report. Note your top 3 apps, number of pickups, the first and last app you use each day, and your mood (1–10) before and after one session. At week's end, choose one value-based change and one friction change. Apply it for a week and compare.

Self-check

1. Explain B = MAP and the Hooked model.
2. Why are variable rewards so habit-forming?
3. What are the ICD-11 features of gaming disorder?
4. Summarise both sides of the smartphone and youth mental-health debate.
5. List five strategies for a client who wants to reduce compulsive phone use.
6. What is technostress? Name its five dimensions.

Go deeper: Your Undivided Attention (CHT) · Attention economy · Dark patterns · FOMO · Gaming disorder · Technostress · US Surgeon General: Loneliness advisory (PDF) · Books: Lembke, *Dopamine Nation*; Newport, *Digital Minimalism*; Haidt, *The Anxious Generation* (read critically)

MODULE 18 · PHASE 3 · WEEK 32

Mental Health in India

Context for all practice

Why it matters: theories mostly come from the West. Your clients live in India, with Indian families, languages, beliefs and systems.

18.1 The numbers

- **National Mental Health Survey 2015–16** (NIMHANS, for MoHFW; ~39,500 adults across 12 states): about **10.6%** of adults had a current mental disorder; **13.7%** lifetime. Roughly **150 million** Indians were estimated to need care.
- **Treatment gap**: the summary reports a gap of roughly **28–83%** across disorders, and about 86% for alcohol use disorders. Most people who need care don't get it.
- Suicide: NCRB reported about **1.7 lakh** suicides in 2022; family problems and illness are the commonest recorded causes. Student and daily-wage-worker suicides are a recognised concern.
- The mental health workforce is far below need, especially outside cities. Counsellors are a vital part of closing the gap.

18.2 The system

Programme / body	What it is
Tele-MANAS (launched Oct 2022)	National tele-mental-health service: **14416** or 1-800-891-4416. Free, 24×7, many languages, with referral to in-person care.
National Mental Health Programme (1982) / District Mental Health Programme	Community-level services through district hospitals
Ayushman Bharat Health & Wellness Centres	Include mental health in primary care packages
National Suicide Prevention Strategy (2022)	India's first; aims to reduce suicide mortality by 10% by 2030
NIMHANS (Bengaluru)	Apex mental-health institute: training, research, specialty clinics (e.g. SHUT Clinic)
NGOs and helplines	e.g. **iCall** (TISS), Sangath, Vandrevala Foundation, The Banyan, MPower
Private and digital sector	Therapy platforms, EAP providers, workplace wellness companies, apps

18.3 Culture and counselling in India

- **Stigma:** fear of being called "pagal", impact on marriage prospects and family reputation. People may first go to a GP, a faith healer or an astrologer. Respect beliefs while ensuring safety and effective care.
- **Family:** decisions are often collective. Involving family (with consent) can help. Individual autonomy may need to be balanced with family harmony (the interdependent self, Module 10).
- **Somatisation:** distress is often expressed physically ("tension", headaches, "weakness", gas, body pain). Ask about the body as well as feelings.
- **Language:** many emotions are easier to express in one's mother tongue. Counselling in Hindi or regional languages is a valuable skill.

- **Religion and spirituality:** often a source of coping and meaning. Yoga, meditation and prayer can complement counselling.
- **Diversity and power:** caste, class, gender, religion, region and sexuality shape distress (minority stress, discrimination). Practise cultural humility; examine your own position.
- **Gender:** men are often discouraged from showing emotion ("mard ko dard nahi hota") and may seek help late. Women face caregiving burdens, safety concerns and domestic violence.
- **Money:** session fees are a barrier. Consider sliding scales, group sessions, and referral to free services.
- **Indigenous psychology:** concepts like *dharma*, *karma*, *sthitaprajna* (equanimity) and *seva* can be culturally resonant frames, used with care and never imposed.

Try this

Write down three things your own family believes about mental health (helpful or not). How would those beliefs affect you if you needed help? How might they show up in clients?

Self-check

1. What did the NMHS 2015–16 find about prevalence and treatment gap?
2. What is Tele-MANAS, and what's its number?
3. Give four cultural factors that shape counselling in India.
4. What is somatisation and why does it matter here?

Go deeper: National Mental Health Survey (NIMHANS) · NMHS summary (MoHFW PDF) · Tele-MANAS · NIMHANS · iCall (TISS) · Mental health in India

MODULE 19 · PHASE 3 · WEEK 33

Looking After the Helper

Professional sustainability

Why it matters: you'll be listening to pain for a living while holding a day job during training. Self-care is a professional requirement, not a luxury.

19.1 Occupational risks for helpers

- **Compassion fatigue**: emotional exhaustion from caring for suffering people.
- **Vicarious (secondary) trauma**: absorbing clients' traumatic material, which changes your own view of the world.
- **Burnout**: yes, counsellors get it too (Module 16).
- **Compassion satisfaction**: the protective positive side, the meaning from helping. Nurture it.

19.2 What protects helpers

- Regular **supervision** and peer consultation.
- **Personal therapy**: many programmes expect or encourage it; it's also the best way to learn what the client's chair feels like.
- **Boundaries**: session limits, caseload limits, no client contact on personal channels, a "commute ritual" to leave work at work.
- **Basics**: sleep, movement, food, time in nature, non-screen hobbies.
- **Connection**: relationships outside the helping role.
- **Meaning**: remember why you started (your Week 1 notebook entry).
- **Self-compassion** (Kristin Neff): self-kindness, common humanity, mindfulness. Treat yourself as you'd treat a client.

19.3 Your personal self-care plan (fill in)

Area	What I'll do	Warning sign I'll watch for
Body (sleep, movement, food)		
Mind (rest, boundaries, learning)		
Relationships		
Meaning / spirit		
Work–study balance		
Who I'll reach out to if I'm struggling		

Self-check

1. Distinguish compassion fatigue, vicarious trauma and burnout.
2. Name five protective practices for helpers.
3. What are Neff's three components of self-compassion?

Go deeper: Compassion fatigue · Vicarious trauma · self-compassion.org (Kristin Neff)

PART D

Practice

Case vignettes, role-play scripts, self-exercises and journal prompts. The vignettes are fictional composites written for learning.

D1. How to use the cases

For each case: read it, answer the questions in your notebook *before* reading the "Points to consider", then compare. There's no single right answer. The goal is to practise thinking like a counsellor: **understand → assess risk → conceptualise → choose an approach → know your limits**.

Case 1 · Priya, 26: "I can't switch off"

Priya is a software engineer in Pune. For four months she's been sleeping 5 hours, checking Slack until midnight, and feeling her heart race on Sunday evenings. She snaps at her mother on calls and has stopped going to dance class. "Everyone else manages. I'm just weak."

Questions: (1) Which lenses (Module 1) apply? (2) Write a 5 Ps formulation. (3) Which thinking traps do you hear? (4) Is this burnout, anxiety, or both? What would you screen for? (5) Two interventions you'd consider.

Points to consider: Biological (sleep loss, arousal), behavioural (checking reinforced by relief: negative reinforcement), cognitive ("I'm weak" = labelling; "everyone else manages" = mind reading or comparison), social (work culture, family). Screen with GAD-7, PHQ-9 and burnout questions; ask about risk. Interventions: psychoeducation on stress and JD-R, sleep hygiene plus a phone-out-of-bedroom experiment, behavioural activation (return to dance), thought records on "weak", boundary-setting scripts. Validate first: her reaction makes sense given her demands.

Case 2 · Rahul, 31: the reluctant engineer

Rahul's family chose engineering for him. He's now a team lead, well paid, and feels "empty". He's considering quitting to teach, but "my parents sacrificed everything; how can I throw it away?"

Questions: (1) Which Erikson stage and Marcia identity status fit? (2) Which approach suits him: person-centred, existential, MI, career counselling? (3) What cultural factors matter? (4) What is your own reaction, given your own career change (countertransference)?

Points to consider: Identity foreclosure revisited at 31; generativity questions; sunk-cost reasoning; the interdependent self (family harmony matters, so the goal may be a path his family can understand, not defiance). Existential and person-centred exploration of meaning, Super's life roles, Schein's anchors, and MI for ambivalence. **Watch your own bias:** don't push him toward change because *you* are changing.

Case 3 · Anjali, 22: "Just tell me what to do"

A final-year student wants advice: break up with her boyfriend or not. She cries, then says, "Sometimes I think everyone would be better off without me."

Questions: (1) What must you do *immediately*? (2) How would you ask? (3) What are the limits of confidentiality here?

Points to consider: "Better off without me" is a possible warning sign (Module 11.4). **Ask directly** about suicidal thoughts, plans, means, past attempts and protective factors. Don't move on to "the boyfriend question" first. If there's risk: don't leave her alone, involve a supervisor, connect to Tele-MANAS (14416) or emergency care, and involve her support people. Confidentiality doesn't cover serious risk; explain this kindly. As a trainee, you never handle this alone.

Case 4 · Vikram, 38: after the layoff (workplace)

Vikram, a product manager, was laid off in a "restructuring" announced over a 10-minute video call. He's sending 30 applications a day, not sleeping, and says, "AI is taking jobs like mine anyway. I'm finished at 38."

Questions: (1) Primary and secondary appraisals? (2) Problem-focused vs emotion-focused coping needs? (3) Which thinking traps? (4) What role could SFBT play?

Points to consider: Threat appraisal plus low perceived resources; identity loss (existential); catastrophising and fortune telling. Balance practical help (structured job-search plan, skills inventory, realistic AI-impact info) with emotion-focused support (grief for the role, sleep, exercise, social contact). SFBT: "When in the past did you get through something hard? What helped?" Scaling: "Where are you on 0–10 in feeling you can cope?" Screen for depression and risk; middle-aged men often hide distress.

Case 5 · The team that stopped talking (organisational)

An HR head asks you to "fix" a design team: two resignations, missed deadlines, nobody speaks in retros. The new director publicly criticises mistakes.

Questions: (1) Which concepts explain this? (2) Who is your client: the team, the individuals, or the company? (3) What ethical issues arise? (4) Interventions at individual, team and leadership levels?

Points to consider: Low psychological safety, groupthink and silence, JD-R (high demands, low support), effort–reward imbalance. Clarify the contract: individuals' confidential disclosures can't go to HR. Interventions: anonymous pulse survey (you know research design), manager coaching on feedback, team norms, EAP referrals for individuals, and evaluating outcomes (Module 15). Your UX research skills fit this naturally.

Case 6 · Arjun, 19: gaming all night (digital)

Arjun's parents bring him in. He games 8–10 hours a day, has failed two semesters, and gets angry when the Wi-Fi is switched off. He says gaming is the only place he has friends.

Questions: (1) Does this fit ICD-11 gaming disorder? (2) What need does gaming meet? (3) How would MI help? (4) When would you refer?

Points to consider: Impaired control, priority over other activities, continuing despite harm, significant impairment: possibly yes, though diagnosis is for qualified clinicians, and you should check duration. Need: belonging, competence, escape (SDT!). MI: explore ambivalence; don't argue. Family sessions; replace rather than remove (offline belonging). Screen for depression, social anxiety and ADHD. Refer to a specialist service (e.g. NIMHANS SHUT Clinic) if severe.

Case 7 · Meera, 34: the always-on mother (digital + work)

Meera works remotely in marketing. Work WhatsApp groups run till 11 pm. She scrolls Instagram "to relax" and feels worse after seeing friends' holidays. "I'm never fully anywhere: not at work, not with my kids."

Questions: (1) Which concepts apply (technostress, social comparison, boundary theory)? (2) Design a 2-week experiment with her. (3) What systemic factors should you acknowledge?

Points to consider: Techno-invasion; upward social comparison; Instagram as avoidant coping. Experiment: notification-free hours, a separate work device or profile, a negotiated team norm for response times, phone-free dinner, replacing evening scrolling with a valued activity (ACT values). Acknowledge gendered caregiving load and workplace norms. Not all of this is hers to fix.

Case 8 · Integration: Sameer, 29

Sameer, a UX designer, wants to become a counsellor but feels like an impostor: "Who am I to help others when I'm still figuring myself out?" He's anxious about money and his parents' opinion.

Questions: Write a complete mini-formulation: 5 Ps, thinking traps, which Erikson stage, impostor phenomenon, SDT needs, two approaches you'd use, and three concrete next steps. *(Yes, this one's partly a mirror. Notice what comes up for you.)*

D2. Role-play scripts (practise with a trusted friend)

Agree first: your friend plays a *made-up* problem (not a real crisis), 10 minutes, then 5 minutes of feedback. Use only Module 13 skills: no advice.

Script	Friend plays…	You practise…
1. Opening	Someone stressed about a presentation tomorrow	Welcoming, open questions, paraphrase, reflection of feeling, summary
2. Silence	Someone who answers briefly and goes quiet	Tolerating silence, encouragers, gentle open questions
3. Goals	Someone wanting to "be less stressed"	Scaling question, miracle question, turning a vague wish into a SMART goal
4. Ambivalence	Someone who wants to cut phone use but loves it	OARS, reflecting both sides, eliciting change talk

Feedback questions for your friend: When did you feel most heard? Did I interrupt or advise? Did I get your feeling right? What did I miss?

D3. Self-exercises collection

Exercise	Module	Time
Six-lens analysis of a personal struggle	1	15 min
One-week sleep log	2	2 min/day
Habit map (cue → behaviour → reward)	4	15 min
Blank-page recall of last week's module	5	10 min
Bias spotting in a recent decision	6	15 min
Stress-and-coping map	7	20 min
Lifeline with Erikson stages	8	30 min
Big Five self-assessment + reflection	9	20 min
Attribution reversal journal	10	5 min/day
PHQ-9 and GAD-7 on yourself	11	5 min
Thought records (×3)	12	15 min each
ABCDE on an upset	12	15 min
Miracle question on your career change	12	15 min
Reflective listening in 3 conversations	13	through the week
Ethical dilemmas	14	20 min
Design a mini-study	15	30 min
JD-R audit of your job + Holland code	16	30 min
7-day digital audit	17	5 min/day
Family beliefs about mental health	18	15 min
Personal self-care plan	19	30 min

D4. Journal prompts (one a week, if you like)

1. Why do I want to help people, really? What's in it for me?
2. Who listened to me well when I needed it? What did they do?
3. What kinds of people or problems would be hard for me to sit with?
4. What did my family teach me about feelings?
5. When did I last change my mind about something important?
6. What does "standing on your own feet" mean to me?
7. How do I usually cope when overwhelmed?
8. What judgements do I make quickly about people?
9. Which counselling approach feels most like "me"? Why?
10. What scares me about this career change?
11. What am I learning about myself from this study?
12. How has my phone shaped my attention this month?
13. What does good work look like to me?
14. What would I want a client to feel after a session with me?
15. What will I protect, in my own life, while I train?
16. Letter to myself at the end of the MA.

PART E

Admission

Checks before you apply, the IGNOU process step by step, alternatives, your statement, interview prep, and your Year-1 routine.

E1. Before you apply: the critical checks (Weeks 25–27)

> **The single most important check:** Will a **distance / ODL** MA Psychology (like IGNOU's) be accepted for **NCAHP registration** as a counselling psychologist? As of October 2026, reports say eligibility depends on qualification, duration, **mode of learning** and experience, and that the position on distance degrees is **unclear**. Read the official notification and ask the NCAHP or your State Allied & Healthcare Council in writing before you pay fees.

☐	Check	Where
☐	NCAHP: is ODL acceptable? Any practicum-hour requirement?	ncahp.abdm.gov.in · ahpr.abdm.gov.in/faq
☐	IGNOU MAPC: current fee, eligibility, July 2027 dates, practical-session locations	ignou.ac.in/schools/programme/MAPC · ignouadmission.samarth.edu.in
☐	UGC-DEB recognition of the specific programme	UGC DEB website (approved programmes list)
☐	Your nearest IGNOU regional centre and study centre that runs MAPC practicals	IGNOU site → Regional Centres
☐	Talk to 2–3 IGNOU MAPC students or alumni (LinkedIn, Reddit, Telegram groups)	Ask: workload, practicals, quality of counselling specialisation, internships
☐	Talk to 2–3 practising counselling psychologists	Ask: how they got supervised hours, first clients, what they'd do differently

If NCAHP doesn't accept ODL degrees: Plan B

- **Regular part-time, evening or weekend MA/MSc programmes** in your city (some private and state universities offer these).
- **A regular MSc Counselling Psychology**. Many require a psychology paper at UG; some accept a **PG Diploma in Counselling** or counselling work experience instead (e.g. St Aloysius, Mangalore). A PG Diploma could be your bridge.
- **Do IGNOU anyway for the knowledge**, then a regular programme later. Costs more time, but your foundation would be excellent.

E2. IGNOU application: step by step

1. **Watch for the notification** (July sessions usually open around May–June; deadlines often extend into July or August). Check the official site; ignore third-party "agents".
2. **Read the Common Prospectus / Programme Guide** for MAPC: eligibility (a bachelor's in any discipline), fee, duration (min 2, max 5 years), course structure.
3. **Prepare scans**: recent photo, signature, **degree certificate** and marksheets, ID (Aadhaar), category certificate if applicable. Check size and format limits on the portal.
4. **Register** on the admission portal with your email and mobile number. Keep the username and password safe.
5. **Fill the form**: programme MAPC, **regional centre** and **study centre** (choose one that conducts MAPC practicals and is convenient), and personal and educational details.
6. **Pay the fee online** (first-year fee + registration fee). Save the receipt.
7. **Track status**: documents verified → enrolment number issued. Save the confirmation.

8. **Study material**: printed blocks and/or eGyanKosh digital access.
9. **Assignments (TMAs)**: one per course, submitted by the session deadline. Assignments usually carry about 30% weight and the term-end exam about 70% (confirm in the Programme Guide).
10. **Practicals (MPCL-007 in Year 1)**: attend the scheduled sessions at your study centre. Plan leave from work in advance.
11. **Re-register for Year 2** and choose the **Counselling Psychology specialisation** (MPCE-021 Counselling Psychology, MPCE-022 Assessment in Counselling & Guidance, MPCE-023 Interventions in Counselling, MPCE-024 Practicum, MPCE-025 Internship, MPCE-026 Project, per IGNOU's programme page).

E3. MAPC structure at a glance

Year	Code	Course	Where this guide prepares you
1	MPC-001	Cognitive Psychology, Learning & Memory	Modules 3–6
	MPC-002	Life Span Psychology	Module 8
	MPC-003	Personality: Theories & Assessment	Module 9
	MPC-004	Advanced Social Psychology	Module 10
	MPC-005	Research Methods in Psychology	Module 15
	MPC-006	Statistics in Psychology	Module 15
	MPCL-007	Practicals	Modules 9, 11, 15
2 (Counselling)	MPCE-021	Counselling Psychology	Modules 12–14
	MPCE-022	Assessment in Counselling & Guidance	Modules 9, 11, 16
	MPCE-023	Interventions in Counselling	Modules 12, 16, 17
	MPCE-024	Practicum in Counselling Psychology	Module 13, Part D
	MPCE-025	Internship	Modules 13, 14, 19
	MPCE-026	Project	Module 15 (your research edge)

Course list from IGNOU's MAPC programme page and 2026–27 assignment files. Confirm against the current Programme Guide. IGNOU sometimes revises codes and credits.

Tip: past question papers for every course are free on IGNOU's website. In Phase 3, attempt two MPC-001 questions under exam conditions; you'll be surprised how much you can answer.

E4. Statement of purpose (for any programme, scholarship or internship)

IGNOU doesn't usually ask for one, but regular programmes, internships and future jobs will. Draft it in Week 34 while your reasons are fresh.

Paragraph	Content	Your material
1. Hook	A specific moment that shows why mental health matters to you	Long-held interest; the AI-era need you see
2. Background	Engineering → UX; how it connects	User research, empathy, human factors (HFI CUA)
3. Preparation	What you've done to prepare	36-week self-study, PFA certificate, volunteering, reading
4. Focus	What you want to specialise in, and why	Workplace counselling + digital-age wellbeing
5. Fit & goals	Why this programme; 5-year plan	Hybrid transition; design-informed counselling
6. Close	What you'll contribute	A bridge between technology and human care

Keep it to about 600–800 words, specific not grand, honest about being a career-changer (it's a strength).

E5. Interview questions to prepare (regular programmes and internships)

1. Why psychology, why now, and why counselling?
2. How does your UX background help or hinder you?
3. What have you done to prepare?
4. Which counselling approach appeals to you most, and why?
5. What is the difference between counselling and advice?
6. How would you handle a client who talks about suicide?
7. What are the limits of confidentiality?
8. Tell us about a time you listened to someone in distress.
9. How do you look after your own mental health?
10. What issues in Indian mental health concern you?
11. How will you manage work and study together?
12. Where do you see yourself in five years?

E6. Your Year-1 weekly routine (once enrolled)

Block	Hours/week	Notes
Course blocks (eGyanKosh / printed)	4–5	6 courses: roughly one block per course every 2–3 weeks
Assignments (TMAs)	1–2	Start each one early; aim to submit well before the deadline
Listening (keep the habit)	1–2	Podcasts linked to the current course
Exam prep (last 6 weeks)	+3	Past papers under timed conditions
Total	**~7–10**	Up from 3–5 now. Build towards this in Phase 3.

IGNOU suggests roughly 30 study hours per credit. Year 1 carries roughly 30 credits (6 four-credit courses plus practicals; check the Programme Guide), so following it fully would mean ~900 hours/year. Most working students do less and focus on assignments and exams. Your 9-month head start makes the gap much smaller.

PART F

Reference

Answers to the self-checks, a glossary, the complete resource directory, and sources.

F1. Self-check answers (short form)

Brief model answers. For fuller answers, re-read the module. Try answering yourself first!

M1. (1) Counselling: everyday distress, adjustment, growth; clinical: assessment and treatment of serious disorders (RCI in India); psychiatry: medical, prescribes. (2) Wundt, Leipzig, 1879. (3) Biological (body/brain), behavioural (learning), cognitive (thoughts), psychodynamic (unconscious/past), humanistic (growth/meaning), socio-cultural (context). (4) Problems arise from interacting biological, psychological and social factors. (5) One study can be a fluke; repeated findings are trustworthy.

M2. (1) Sympathetic: fight/flight; parasympathetic: rest/digest. (2) Amygdala; prefrontal cortex. (3) Hypothalamus → pituitary → adrenals release cortisol under stress. (4) Evidence doesn't support a simple chemical cause; depression is biopsychosocial. (5) CBT-I.

M3. (1) Sensation = detecting; perception = interpreting. Bottom-up = data-driven; top-down = expectation-driven. (2) Proximity, similarity, closure, continuity (also figure–ground, common fate). (3) Missing obvious things when attention is elsewhere; Simons & Chabris's "invisible gorilla". (4) Pulls attention toward threat cues.

M4. (1) UCS naturally triggers a response; CS was neutral and acquires the power through pairing. (2) Removing something unpleasant increases behaviour; avoidance brings relief, so avoidance grows and the fear is never disproven. (3) Variable ratio, e.g. pull-to-refresh, loot boxes. (4) A temporary increase when reinforcement stops. Warn clients so they don't give up. (5) Attention, retention, reproduction, motivation.

M5. (1) Central executive, phonological loop, visuospatial sketchpad, episodic buffer. (2) Deeper (semantic) processing → better memory, e.g. linking a word's meaning to your life vs noting its font. (3) Episodic = personal events; semantic = facts; procedural = skills. (4) Question wording altered speed estimates and created false memories, so use non-leading questions. (5) Mood makes matching memories easier to recall.

M6. (1) Algorithm = step-by-step, guaranteed (a recipe); heuristic = shortcut (rule of thumb). (2) Availability: "I'm next to be laid off" after news; confirmation: noticing only evidence that "nobody likes me". (3) Fluid = novel reasoning; crystallised = accumulated knowledge. (4) Structure of intellect (operations × contents × products); divergent = generating many possible answers. (5) Preparation, incubation, illumination, verification.

M7. (1) Performance is best at moderate arousal; the optimum is lower for complex tasks. (2) Autonomy, competence, relatedness. (3) J-L: body → emotion; C-B: simultaneous; S-S: arousal + label. (4) Primary: is it a threat? Secondary: can I cope? (5) When the situation is controllable.

M8. (1) Sensorimotor (object permanence), preoperational (symbolic thought), concrete (conservation), formal (abstract reasoning). (2) What a learner can do with help but not alone. (3) Identity vs role confusion; intimacy vs isolation; generativity vs stagnation; integrity vs despair. (4) Secure, anxious-avoidant, anxious-resistant/ambivalent, disorganised. (5) Grief isn't a fixed sequence; it varies by individual and culture.

M9. (1) Id = drives; ego = mediator; superego = conscience. Defences: repression, denial, projection, displacement, rationalisation (+ others). (2) Conditions of worth: love felt as conditional; incongruence: a gap between self-concept and experience. (3) Mastery, vicarious experience, persuasion, physiological state. (4) OCEAN; MBTI has poor reliability and false categories. (5) Sattva (clarity), rajas (activity), tamas (inertia). (6) Reliability = consistency; validity = measures what it claims.

M10. (1) FAE: over-blaming personality in others; self-serving: credit for success, blame situations for failure. (2) $1 group rated the dull task more enjoyable, reducing dissonance by changing belief. (3) Asch: conformity to peers; Milgram: obedience to authority (~65% to max). (4) Cohesive groups suppressing dissent; prevent with psychological safety, a devil's advocate, outside input. (5) Consider family and role-based goals; involve family where appropriate; independence isn't always the aim.

M11. (1) Deviance, distress, dysfunction, danger. (2) DSM-5-TR and ICD-11; India uses ICD officially. (3) Depression: low mood or anhedonia ≥2 weeks + other symptoms; panic: recurrent unexpected attacks + worry or avoidance. (4) 5/10/15/20; item 9 (self-harm). (5) Asking doesn't increase risk; talkers may act; anyone can help connect. (6) Suicide risk, psychosis, mania, need for medication (also severe depression, withdrawal).

M12. (1) Alliance, empathy, hope, rationale, client resources. (2) Empathy, UPR, congruence. (3) Core beliefs → intermediate beliefs → automatic thoughts; triad = negative self, world, future. (4) Your example. (5) Miracle question as in 12.6; "On 0–10, where are you now?" (6) Acceptance, defusion, present moment, self-as-context, values, committed action. (7) Open questions, Affirmations, Reflections, Summaries; precontemplation, contemplation, preparation, action, maintenance.

M13. (1) Referral, intake, contracting, relationship, conceptualisation and goals, intervention, evaluation, termination. (2) Current picture, preferred picture, way forward. (3) Your examples. (4) They feel accusatory. (5) Presenting, predisposing, precipitating, perpetuating, protective. (6) Counsellor's emotional reactions to the client; notice and take to supervision.

M14. (1) Autonomy, beneficence, non-maleficence, justice, fidelity. (2) Risk to self or others, child abuse, court order, client consent, supervision. (3) Presumes a person attempting suicide is under severe stress; not to be punished. (4) POCSO, 2012. (5) Employers pay, but must not receive individual information.

M14 dilemmas. (a) Don't "counsel" a friend's relative informally. Listen kindly, encourage professional help, share Tele-MANAS. (b) Share nothing without the client's written consent; you can't even confirm they're a client. (c) Decline politely and explain your boundary policy at the next session.

M15. (1) Third variables and direction problems. (2) IV manipulated; DV measured; operational definition = how it's measured; confound = alternative explanation. (3) ~68%. (4) $z = (85-100)/15 = -1$. (5) Type I = false positive; Type II = false negative. (6) ANOVA. (7) Familiarise, code, search themes, review, define and name, write up.

M16. (1) Chronic unmanaged workplace stress; exhaustion, mental distance/cynicism, reduced efficacy. (2) Demands drain, resources energise and buffer. (3) Karasek: high demands + low control = strain; Siegrist: high effort + low reward = stress. (4) Shared belief that it's safe to take interpersonal risks. (5) Employer-funded confidential short-term counselling; no individual data to the employer. (6) RIASEC; growth, exploration, establishment, maintenance, disengagement.

M17. (1) B = MAP: motivation + ability + prompt; Hooked: trigger, action, variable reward, investment. (2) Unpredictability sustains responding (variable ratio). (3) Impaired control, priority over other activities, continuing despite harm, impairment, usually ~12 months. (4) Haidt: phones caused the rise; critics: evidence is small or correlational. (5) Friction, notifications off, phone out of bedroom, replacement activities, values-based use (also MI, sleep hygiene). (6) Stress from technology; overload, invasion, complexity, insecurity, uncertainty.

M18. (1) ~10.6% current, 13.7% lifetime; treatment gap ~28–83% across disorders. (2) National tele-mental-health helpline; 14416. (3) Stigma, family role, somatisation, language, religion, caste/gender/class, cost. (4) Distress expressed as body symptoms; ask about the body, not just feelings.

M19. (1) Compassion fatigue = exhaustion from caring; vicarious trauma = changed worldview from others' trauma; burnout = chronic work stress. (2) Supervision, personal therapy, boundaries, basics (sleep, movement), connection, meaning. (3) Self-kindness, common humanity, mindfulness.

F2. Glossary

ABCDE (REBT)
Activating event, Belief, Consequence, Disputing, Effective new belief.

Acceptance and Commitment Therapy (ACT)
Builds psychological flexibility through acceptance, defusion and values-based action.

Actualising tendency
Rogers' idea of an innate drive toward growth.

Adjustment disorder
Disproportionate distress after an identifiable stressor.

Anhedonia
Loss of interest or pleasure.

Attachment
The emotional bond with caregivers; shapes models of relationships.

Attribution
How we explain causes of behaviour (dispositional vs situational).

Automatic thoughts
Quick, situation-specific thoughts that trigger emotions.

Avoidance
Escaping feared situations; maintained by negative reinforcement.

Behavioural activation
Scheduling rewarding activities to lift mood.

Big Five (OCEAN)
Openness, Conscientiousness, Extraversion, Agreeableness, Neuroticism.

Biopsychosocial model
Biological, psychological and social factors interact.

Burnout
WHO occupational phenomenon: exhaustion, cynicism, reduced efficacy.

Case conceptualisation
A working explanation of a client's difficulties (e.g. the 5 Ps).

Catastrophising
Expecting the worst possible outcome.

Classical conditioning
Learning by association (Pavlov).

Cognitive defusion
Seeing thoughts as passing mental events, not truths.

Cognitive dissonance
Discomfort from conflicting beliefs and actions.

Cognitive distortion
A habitual thinking error (thinking trap).

Cognitive triad
Beck: negative views of self, world, future.

Common factors
Elements shared by effective therapies (alliance, empathy, hope).

Compassion fatigue
Exhaustion from caring for suffering people.

Conditions of worth
Feeling valued only when meeting others' conditions.

Confidentiality
Keeping client information private, with defined limits.

Congruence
Counsellor genuineness (Rogers).

Coping
Efforts to manage stress: problem- or emotion-focused.

Core beliefs
Deep, global beliefs about self, others, world.

Correlation
A statistical relationship (r, from −1 to +1); not causation.

Countertransference
The counsellor's emotional reactions to a client.

CPD
Continuing Professional Development (NCAHP: 15 hrs/yr).

Dark patterns
Deceptive design that steers users against their interests.

Defence mechanisms
Unconscious strategies that protect against anxiety.

Diathesis–stress
Vulnerability + stress → disorder.

DSM-5-TR
American Psychiatric Association's diagnostic manual (2022).

EAP
Employee Assistance Programme: confidential workplace counselling.

Effect size
The magnitude of an effect (e.g. Cohen's d).

Empathy
Understanding another's experience from their frame, and conveying it.

Exposure
Gradual facing of feared situations to reduce anxiety.

Extinction
Weakening of a learned response when reinforcement stops.

FOMO
Fear of missing out.

Fundamental attribution error
Over-attributing others' behaviour to personality.

GAD-7
7-item anxiety screener (0–21).

Gaming disorder
ICD-11 diagnosis of impaired control over gaming with harm.

General Adaptation Syndrome
Selye: alarm, resistance, exhaustion.

Groupthink
Harmony-seeking groups making poor decisions.

HPA axis
Hypothalamus–pituitary–adrenal stress hormone system.

Heuristic
A mental shortcut.

ICD-11
WHO's International Classification of Diseases, 11th revision.

Immediacy
Discussing what's happening between counsellor and client now.

Impostor phenomenon
Feeling like a fraud despite competence.

Informed consent
The client's agreement based on clear information.

JD-R model
Job demands drain; job resources motivate and buffer.

Learned helplessness
Giving up after repeated uncontrollable events.

Locus of control
Belief that outcomes are internally or externally controlled.

MHCA 2017
India's Mental Healthcare Act; rights-based mental health law.

Micro-skills
Basic counselling communication skills (Ivey).

Miracle question
SFBT question imagining the problem solved.

Motivational interviewing
Collaborative style for resolving ambivalence about change.

NCAHP
National Commission for Allied and Healthcare Professions; regulates counselling psychologists.

Negative reinforcement
Removing something unpleasant to increase behaviour.

Neuroplasticity
The brain's ability to change with experience.

Null hypothesis
The assumption of no effect or difference.

OARS
Open questions, Affirmations, Reflections, Summaries.

Operant conditioning
Learning from consequences (Skinner).

Operational definition
How a concept is measured in a study.

Paraphrase
Restating a client's content in fresh words.

PFA
Psychological First Aid: Look, Listen, Link.

PHQ-9
9-item depression screener (0–27).

POCSO
Protection of Children from Sexual Offences Act, 2012.

POSH
Prevention of Sexual Harassment at workplace Act, 2013.

Psychological safety
Team belief that it's safe to take interpersonal risks.

RCI
Rehabilitation Council of India; licenses clinical psychologists.

Reflection of feeling
Naming the emotion underneath what's said.

Reliability
Consistency of a measure.

Resilience
Adapting well in the face of adversity.

Schema
A mental framework organising knowledge and expectations.

Self-efficacy
Belief in one's ability to succeed at a task (Bandura).

Self-Determination Theory
Needs for autonomy, competence, relatedness.

SFBT
Solution-Focused Brief Therapy.

Socratic questioning
Guided questions to help clients examine thoughts.

Somatisation
Expressing psychological distress through physical symptoms.

Stages of change
Precontemplation → contemplation → preparation → action → maintenance.

Standard deviation
Typical spread of scores around the mean.

SUDS
Subjective Units of Distress (0–100).

Supervision

Ongoing review of a counsellor's work by an experienced professional.

Technostress
Stress caused by technology use at work.

Tele-MANAS
India's national tele-mental-health helpline (14416).

Therapeutic alliance
Bond + agreement on goals and tasks between client and counsellor.

Thought record
CBT worksheet for examining automatic thoughts.

Transference
Client relating to the counsellor like a past figure.

Triguna
Sattva, rajas, tamas: Indian model of personality.

Unconditional positive regard
Non-judgemental acceptance of the client.

Validity
Whether a measure assesses what it claims.

Variable-ratio schedule
Rewards after an unpredictable number of responses.

Vicarious trauma
Helper's changed worldview from exposure to others' trauma.

Working memory
Active mental workspace (Baddeley & Hitch).

Yerkes–Dodson law
Performance best at moderate arousal.

z-score
(score − mean) / SD.

Zone of proximal development
What a learner can do with guidance (Vygotsky).

F3. Complete resource directory

Links were gathered in October 2026. Some (marked †) are well-known stable addresses that couldn't be live-tested from this environment. If a link has moved, search for the resource by name.

Free textbooks, courses and references

Resource	Use	Link
OpenStax Psychology 2e	Core textbook	openstax.org/details/books/psychology-2e
OpenStax Introductory Statistics	MPC-006 prep	openstax.org/…/introductory-statistics-2e †
Khan Academy Statistics	MPC-006 prep	khanacademy.org/math/statistics-probability †
Crash Course Psychology	Video overview	YouTube playlist †
Science of Well-Being (Yale/Coursera)	Wellbeing course	coursera.org/learn/the-science-of-well-being †
Introduction to Psychology (Yale/Coursera)	Optional depth	coursera.org/learn/introduction-psychology †
APA Dictionary of Psychology	Look up terms	dictionary.apa.org †
Noba Project	Free modules (login)	nobaproject.com
IGNOU eGyanKosh	MAPC course blocks	egyankosh.ac.in
IGNOU MAPC programme page	Structure, admission	ignou.ac.in/schools/programme/MAPC
IGNOU assignments (2026–27)	MPC-001/002/005/006	001 · 002 · 005 · 006
IGNOU past paper example	Exam practice	MPC-001, June 2024
Psychological First Aid course	Certificate (~2 hrs)	disasterready.org/principles-of-psychological-first-aid
WHO PFA Guide for Field Workers	Free guide	who.int/publications/i/item/9789241548205 †
PHQ-9 / GAD-7	Screening tools	phqscreeners.com †
Big Five test (IPIP)	Self-assessment	openpsychometrics.org/tests/IPIP-BFFM †
Beck Institute	CBT resources	beckinstitute.org †
Self-compassion (Kristin Neff)	Exercises	self-compassion.org †

Podcasts

Podcast	Why	Link
Hidden Brain	Psychology research stories	hiddenbrain.org †
Speaking of Psychology (APA)	Researcher interviews, transcripts	apa.org/news/podcasts/speaking-of-psychology
WorkLife with Adam Grant	Workplace psychology	ted.com/podcasts/worklife †
How's Work? (Esther Perel)	Real workplace sessions	estherperel.com/podcast
Where Should We Begin? (Esther Perel)	Real couples sessions: hear the skills	estherperel.com/podcast
Your Undivided Attention (CHT)	Technology & wellbeing	humanetech.com/podcast

Books

Book	Author	Module
Maybe You Should Talk to Someone	Lori Gottlieb	Phase 1
Man's Search for Meaning	Viktor Frankl	7, 12
Feeling Good	David D. Burns	12
The Gift of Therapy	Irvin Yalom	12–13
On Becoming a Person (optional)	Carl Rogers	9, 12
The Skilled Helper (optional)	Gerard Egan	13
Thinking, Fast and Slow (optional)	Daniel Kahneman	6
Burnout	Emily & Amelia Nagoski	16
Dopamine Nation	Anna Lembke	17
Digital Minimalism	Cal Newport	17
The Anxious Generation (read critically)	Jonathan Haidt	17

India: services, regulation, data

Resource	Link
Tele-MANAS (14416)	telemanas.mohfw.gov.in †
iCall (TISS)	icallhelpline.org
NIMHANS	nimhans.ac.in †
National Mental Health Survey	indianmhs.nimhans.ac.in † · summary PDF
NCAHP	ncahp.abdm.gov.in · enrolment FAQ
IGNOU admission portal	ignouadmission.samarth.edu.in

Workplace and digital

Resource	Link
WHO: burnout as occupational phenomenon	who.int (2019)
WHO/ILO: mental health at work (2022)	who.int (2022) · full guidelines (NCBI)
US Surgeon General: loneliness advisory (2023)	hhs.gov PDF †
Center for Humane Technology	humanetech.com

F4. Sources for career, regulation and money facts

- NCAHP rules: Careers360 · Psychologs · AHPR FAQ · NCAHP (Wikipedia)
- RCI / clinical psychology changes: CLEAR · UPS Education · Careers360: UGC/NCAHP/RCI tangle
- IGNOU fees and structure: GetMyUni · Distance Education School · IGNOU MAPC page · FindMyCollege syllabus summary
- MSc Counselling eligibility: Kristu Jayanti · St Aloysius · SJU
- Counsellor pay: Payscale (early) · Payscale (mid) · TherapyRoute India · SVYASA
- UX pay: Instahyre · Product Leadership
- NEET age limit: Medical Dialogues
- NMHS figures: MoHFW summary · PMC article
- WHO/ILO workplace guidance: ILO news release

> **You started this because you want to help people stand on their own feet.**
> Thirty-six weeks from now you'll walk into your MA already knowing much of what most of your classmates will meet for the first time.
> Go slowly, be kind to yourself, and keep the notebook. 🌱